

WORKING PAPER · VERSION 1.3

The Health Record Rights Index

Who Holds the Record in 198 Countries?

Jason Alan Snyder¹, Robert P. Hill IV¹, Dustin Raney¹, Leann Sims²

¹SuperTruth Inc., United States. ²MedSync, United States. J. A. Snyder ORCID 0009-0001-6157-8100.

Status	Working paper, version 1.3, not peer reviewed. 6 October 2026. Version 1.0 was dated 2 October 2026; versions 1.1 and 1.2 were dated 4 October 2026. The changes are listed after the abstract.
Correspondence	through the SuperTruth contact form, https://supertruth.ai/on-the-record#contact
Submitted	October 2026
Version	1.3
Country data as of	2026-10-02
State law read	2026-10-03
DOI	this version, https://doi.org/10.5281/zenodo.23199733 .
All versions	https://doi.org/10.5281/zenodo.23120174
Version 1.2	https://doi.org/10.5281/zenodo.23137550
Version 1.1	https://doi.org/10.5281/zenodo.23135591
Version 1.0	https://doi.org/10.5281/zenodo.23120175 (data, all versions: https://doi.org/10.5281/zenodo.23120172 ; data version 1.2, which this version uses: https://doi.org/10.5281/zenodo.23174624).
Interactive index	https://healthrecordrights.com
License	The paper and data are released under CC BY 4.0 and the code under the MIT license.

Abstract

We scored 198 countries and territories (all 193 UN member states, plus Greenland, Kosovo, Palestine, Taiwan and Vatican City) from 0 to 100 on one question: does a person get to see, control and share their own health record? Eight weighted categories cover access to the full record, control and consent, privacy and security, protection from commercial use, a connected care journey, clinician access at the point of care, research consent, and clinical AI governance. Research agents built on Anthropic's Claude models did the research and scoring against a written rubric, and the authors ruled on every finding and every change for the countries; Section 11.7 sets out how rulings were made for the US state layer. The rubric requires an agent to open every source it cites; a later check of every cited link found 7156 cited links checked: 7011 opened, 0 dead (HTTP 404 or 410), 88 blocked by bot protection and 57 unreachable, unresolved or answering with a server error. Every score cites the public pages it rests on: 5147 cited sources (979 undated). Finland (71), Denmark (70), Estonia (68), Hungary (68) and Sweden (67) have the highest scores; allowing for scoring error, each of them is first, alone or tied, in at least 5% of draws. The median is 39, and scores track national income closely: Spearman +0.72 (95% interval +0.64 to +0.79, n 188). By band, with the range each count takes when weights and scores are varied: 9 Strong (7 to 14 across draws), 66 Mixed (58 to 72), 111 Weak (101 to 115) and 12 Poor (10 to 19). No country reaches the top band in any draw; 12 countries are Poor as published (10 to 19 across draws). A blind re-scoring of 58 sampled cells from the first 65 countries by a separate session of the same model family, run before the 2 October re-research, agreed with the scores at that time at an intraclass correlation of 0.82, and a second, of 108 cells from the 133 countries added on 2 October 2026, at 0.84, though single access scores there agree less well (0.68); no human rater has scored the index. A single cell can move by about 17 points. Once a country's scoring error and the weights are both allowed for, a country's likely rank typically spans 43 places. Confidence labels are computed from each country file by a fixed rule: 27 high, 105 medium and 66 low. We also classed where control over the record sits by default, which we call who holds the keys: 51 Shared, 81 Institutional and 66 State; 0 Individual. No country gives the person the default say over who sees the record; our definition of that class requires that nothing flow into a state or provider system by default, which no national system does. An audit of our own scores found that the same legal situation was sometimes scored differently in different regions. We tightened the rubric, rescored, and report every change. A separate context layer, which changes no score, asks whether strained public systems push care to private providers whose records do not reach the public record. Alongside the scores we collected 463 published accounts of real problems with health records. They illustrate the scores and do not change them. Version 1.1 adds a layer for the United States: what the law of each of

the 50 states, and of Washington, DC, adds to federal law for a person's health record, on six legal dimensions and four kinds of sensitive record. It gives no state score and no state rank, and it changes no country score. A blind re-reading of 150 sampled cells, by agent sessions that could read only stored copies of the cited pages, agreed with the published cells at an intraclass correlation of 0.965 on the 114 cells with ordered levels where the rater settled a level (a bootstrap interval clustered by state runs from 0.92 to 1.00). Other sessions that searched on their own reported a law the research sessions had not found in 4 of 25 sampled cells, which set off a second search for missed laws. The paper and data are released under CC BY 4.0 and the code under the MIT license.

Keywords: health records, patient access, consent, health data governance, interoperability, European Health Data Space, HIPAA, GDPR, clinical AI governance, composite index, US state law

Changes in version 1.3

Version 1.3 (6 October 2026) brings Section 11 up to date with the state data deposited as data version 1.2, and corrects the conflict of interest statement. No country score, rank, band, keys class or confidence label changed since version 1.2.

- **Conflict of interest corrected.** Version 1.2 said the state layer does not name any exchange. Since the change below, it records the exchange each state designates, where there is one. It still scores no exchange and does not name MedSync. The statement now says so.
- **Exchange consent filled in for every state (D-35).** In version 1.2, four exchange answers were not checked: Alabama, Connecticut, New Hampshire and New Jersey. Each now has an answer with its sources, and the authors ruled on the two that needed a ruling (Section 11.7). States with an opt-out exchange went from 31 to 33, and exchange answers not checked from 4 to 0. Eight more states now name their exchange, or say plainly that none is designated, with no change to their answer.
- **Copy deadlines worded exactly (D-33, D-34).** Florida, Delaware and Texas now say who the faster copy deadline covers. No level changed.
- **Country files.** Since version 1.2, the files for Bangladesh and India were edited in text only: large numbers are written in the international style rather than in crore and lakh. Every analysis that rests on the scores was rerun on the current data.
- **Figures recounted.** The published accounts of real record problems grew from 451 to 463 with the weekly refresh. The link check and the evidence grade (Section 8) are still the run of 3 October 2026,

because no edit since then has changed a cited link; the link results and evidence tiers are those of version 1.2.

- **Data availability.** The state layer is now in the public export and in the Zenodo data record (Data and Code Availability).

Changes in version 1.2

Version 1.2 (4 October 2026) adds Leann Sims, Chief Executive Officer of MedSync, as an author, and updates the Conflict of Interest, Author Contributions and About the Authors sections to match. No score, finding or other text changed.

Changes in version 1.1

Version 1.1 (4 October 2026) adds Section 11, on the 50 states and Washington, DC, with Tables 11 to 13 and Figure 11. It updates the abstract, the use-of-AI statement in Section 3, the limitations, the conflict of interest, the intended use and the data statement to cover the new layer. Sections 11 and 12 of version 1.0 are now Sections 12 and 13. No country score, rank, band, keys class or confidence label changed since version 1.0. The file for the United States was edited in text only and its sources, on 3 October 2026, after a federal law audit and before the main state run began: the US cells now describe the information-blocking rule and the FTC's breach rule (Section 7.3). The main state run did not touch the country data: the hash of the country files was the same before and after it (its first eight characters are 8b590f01). The published accounts grew from 446 to 451, and the counts drawn from them and from the link and evidence checks were recomputed. The limits that version 1.0 named for this version (Section 7.4, item 11) are still open. The global context strip planned in the version 1.1 plan (context figures for every country, like Table 13 for the states) is not included, because its data have not been collected; it is planned for version 1.2. No human inter-rater study has been done.

1. Why this question

A health record follows a person through every part of care: the family doctor, the hospital, the lab, the pharmacy, the insurer, public health and research. Each party in that chain sees its own part. Clinicians see the records their own service holds. Insurers see claims. Governments see registries. In our view the person whose record it is often sees least: a portal, a summary, or a photocopy that took weeks to arrive. The index measures how far that is true, country by country.

Clinical AI raises the stakes. Systems that read records to suggest diagnoses, summarize notes or triage patients are only as good as the record they read. In our view, which this paper does not test, the person whose record it is usually has the least say in how it is used, and the least ability to find out what it contains.

We wanted one comparable answer, country by country, to a plain question: does a person get to see, control and share their own health record? Most existing indices measure the digital maturity of health systems, as reported by governments. The Euro Health Consumer Index and an earlier patient empowerment report scored record access as one item among many (Section 6). We set out to measure the patient’s side as the object of the index: the right in law and, where published evidence allows, whether it works in practice. Law alone is not enough: England’s care.data program failed although it was lawful, because it lacked public trust [52]. We did not measure practice directly: no record requests, patient surveys or tests.

2. What we measured

2.1 Eight categories

Each country receives a score from 0 to 100 in eight categories. The overall score is their weighted average. The weights are the authors’ judgment. Access and control carry the largest printed weights because they are the core of the question. Printed weights are not the same as influence on the ranking: Table 1b, after the category definitions, shows how much each category actually moves the overall score.

Table 1. The eight categories, their weights, and the distribution of scores across all countries (minimum, quartiles, median, maximum).

Category	Weight	Min	Q1	Median	Q3	Max
Patient access to the full record	20% 	15	35	43	60	80
Patient control and consent	20% 	20	28	30	47	74
Privacy and security	15% 	15	35	45	52	65
Connected care journey	15% 	18	28	38	58	82
Protection from commercial use	10% 	15	35	45	52	70
Clinician access at the point of care	10% 	15	23	34	54	78
Research and trial consent	5% 	30	45	46	49	70
Clinical AI governance	5% 	15	22	30	50	66

1. **Patient access to the full record.** The legal right to see and copy the whole record (notes, labs, imaging reports, history), and the practical reality: a national portal or not, summary or full record, how many years, cost and time limits, machine-readable export.

2. **Patient control and consent.** Whether the person decides who sees and uses their data, with granular and revocable choices; whether they can see who accessed it; whether the default is individual or institutional control.
3. **Privacy and security.** Health data's legal status, breach notification, penalties, regulator independence and enforcement, major breaches, state access exceptions.
4. **Connected care journey.** Infrastructure that links the record across primary care, hospitals, labs, pharmacy, claims, public health and research, and how much it is used.
5. **Protection from commercial use.** Limits on selling health data or using it for marketing, data brokers, consumer health apps outside health law, sale of de-identified data. A high score means strong protection.
6. **Clinician access at the point of care.** Whether the treating clinician can see the patient's complete record across providers when needed, under consent and emergency rules.
7. **Research and trial consent.** Ethical, consent-based secondary use; transparency; trial consent law. A high score means the person's choice is respected and research is still possible.
8. **Clinical AI governance.** Rules for AI used in care: device regulation, change control, transparency, data provenance, human oversight, rules for generative AI.

The eight categories move together. Across all 198 countries the first principal component of the eight scores carries 70% of their variance. Table 1b gives each category's share of the variation in overall scores: its weight times its covariance with the overall score, divided by the variance of the overall score, so the shares sum to 100%. Access and control together account for 43.6%. The connected care journey and clinician access, which measure infrastructure, account for 31.6%, so the ranking is driven by infrastructure about as much as by access. Research consent, printed at 5%, accounts for 0.7%: it contributes almost nothing to the ranking (Section 4.4). We keep the eight categories separate as a reporting choice, so that a reader can see each one, not as eight independent pieces of evidence.

Table 1b. Printed weights against each category's share of the variation in overall scores, with the standard deviation (SD) and interquartile range (IQR, from Table 1) of each category across all countries (analysis/variance/variance.py).

Category	Printed weight	Share of the variation in overall scores	SD	IQR
Patient access to the full record	20%	22.1%	15.1	25
Patient control and consent	20%	21.5%	15.0	19
Privacy and security	15%	11.3%	11.7	17
Connected care journey	15%	19.0%	17.6	30
Protection from commercial use	10%	8.6%	12.5	17
Clinician access at the point of care	10%	12.6%	17.7	31
Research and trial consent	5%	0.7%	4.3	4
Clinical AI governance	5%	4.3%	13.7	28

2.2 Bands

Scores map to five bands, anchored in the rubric: **Leading** (85 to 100): the right exists in law and works in practice at national scale, with controls the patient can see. **Strong** (65 to 84): the right exists and mostly works, with notable gaps. **Mixed** (45 to 64): partial rights or partial infrastructure. **Weak** (25 to 44): rights mostly on paper, or a fragmented record. **Poor** (0 to 24): no meaningful right or infrastructure, or active misuse. "Works in practice" is read from published evidence; we did not measure practice directly (Section 1).

2.3 Who holds the keys

Separately from the score, each country is classed by where control sits by default. The classes were given written anchors on 2 October 2026 (docs/RUBRIC_V1_1_ANCHORS.md) and applied to every country in order:

- **Individual:** the record is held or directed by the person, nothing flows into a state or provider system by default, and every disclosure needs the person's prior, revocable consent, with a log the person can read.
- **Shared:** the system shares by default or by law, but the person has at least one working choice over who sees the record in care (opt-in, opt-out, or blocking named providers or items), in force for the general population. An access log alone is visibility, not control, and a research opt-out alone does not count.
- **State:** no such working choice, and a live government-run national record or exchange used for care holds the data by default.
- **Institutional:** no such working choice, and records are held provider by provider with no live national record of that kind.

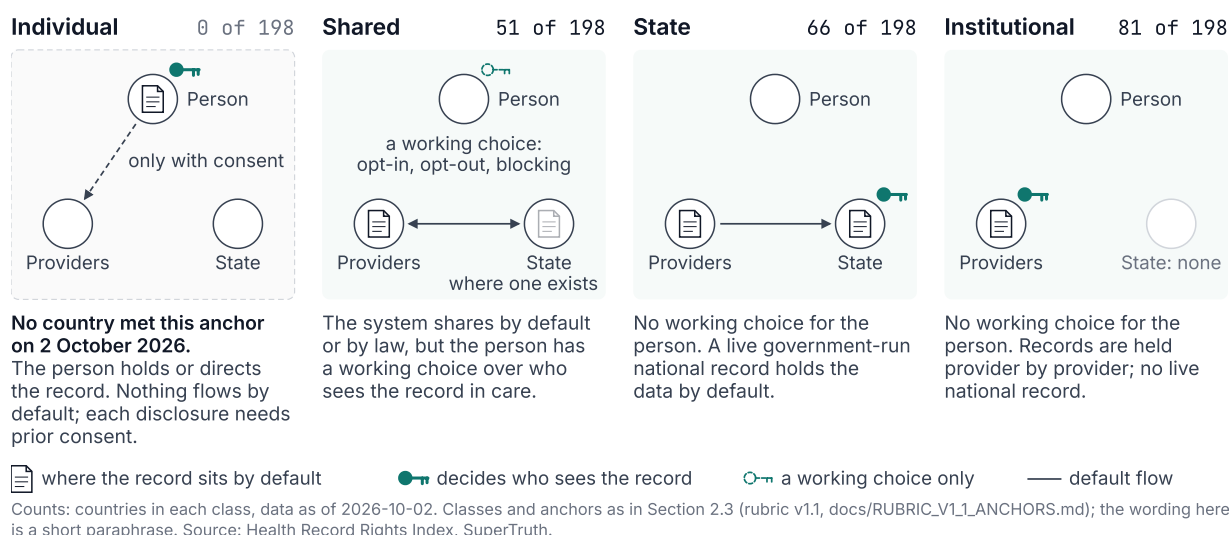


Figure 1. Who holds the keys: the four classes of Section 2.3, drawn as the person, providers and the state. The document marks where the record sits by default, a solid key who decides who sees it, a dashed key a working choice only, and the arrow which way the record flows by default. No country met the Individual anchor.

Applying the anchors in the review round of 2 October 2026 changed Cyprus (Shared to State), Greece (Shared to State) and Indonesia (Shared to State); the re-research of the first 43 countries the same day changed 6 more: Argentina (Institutional to Shared), United Kingdom (State to Shared), Ireland (Institutional to State), Japan (Institutional to Shared), Kenya (State to Shared) and New Zealand (Institutional to Shared). The class and the control score measure different things and can disagree: a State country with a strong log the patient can see can outscore a Shared country with a weak choice.

2.4 What “rights” means here

In the title, rights means what a person is entitled to over their own record and whether they can use it. Five categories score entitlements in law: access, control, privacy, protection from commercial use and research consent. Three score what a state or health system has built: the connected care journey, clinician access and clinical AI governance. We call those three delivery. They are in the overall score because a right a person cannot use is worth less, and the OECD’s own country survey reports health data infrastructure and governance together [59]. The line is not clean: access and control also give credit for a national portal or a consent register, which are infrastructure. Section 4.6 reports a rights sub-score and a delivery sub-score separately, with the income link of each. They are a supplementary decomposition for this paper; the interactive index and Table 2 use the overall score.

3. How we scored

1 Research

One agent per country, in the country's own languages; 198 countries.

Public sources only: laws, government and regulators, court decisions, published news.

The rubric requires every cited source to be opened.

Claude Opus 5.5 through Claude Code, 30 September to 2 October 2026.

Output: a draft country file with 8 category scores, 0 to 100, each with a summary and dated sources.

→ 2 Cross-check

A second agent session, same model family.

Rubric anchors: each score compared with the anchor band for its legal situation.

Publisher classes: every cited publisher reclassified.

→ 3 Review and audit

Agent consistency audit: countries with the same legal right compared.

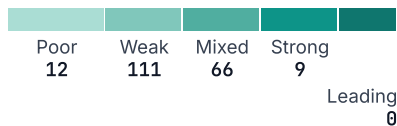
The authors rule on every finding and every proposed score change.

Published change log: every change, with its reason.

4 Scoring

Overall score: the weighted sum of the eight category scores; weights sum to 100% (Table 1).

Five bands, to scale; countries in each:



→ 5 Testing

Stability: 10,000 draws re-weight the categories and perturb the scores; a rank range for each country.

Second full reading of 43 countries: ICC 0.94 on the overall score, 0.86 per cell.

Blind re-scoring of 64 sampled cells (58 scorable): ICC 0.82.

Blind re-scoring of 136 cells from the added countries (108 scorable): ICC 0.84.

Comparison with outside measures (Section 6).

Links: 7,011 of 7,156 cited links open on recheck, 2 October 2026 (US Eastern time).

→ 6 Publication

Scores and every source, as CSV and JSON, CC BY 4.0.

198 one-page country briefs.

The paper.

Beside the scores, never in them: 446 published accounts from regulators, courts and the news. They change no score.

Method as in Section 3. Rubric v1.1. 198 countries, data as of 2026-10-02. Counts read from out/facts.json, analysis/robustness, analysis/reliability, analysis/reliability2, analysis/testretest and analysis/url_check.csv. Source: Health Record Rights Index, SuperTruth.

Figure 2. How the index was built, in six stages. Every count in the figure is read from the repository (scripts/fig_method_print.js).

Countries. 198 countries and territories across 6 regions: all 193 UN member states, plus Greenland, Kosovo, Palestine, Taiwan and Vatican City. The first 43 were chosen to cover every inhabited region and the largest health systems; they were researched in the index's first version and re-researched in full on 2 October 2026. 21 countries were added on 1 October 2026, mostly to complete the European Union, and Albania on 2 October. On 2 October 48 more were added, then 81, then Palestine and Vatican City, so that the index covers every UN member state, and last Greenland and Kosovo, included because each runs its own health system. Greenland, part of the Kingdom of Denmark, makes its own health law, such as a 2001 ordinance on patients' rights and a 2024 act of its parliament, Inatsisartut, on patient complaints and compensation; the Danish Act on Greenland Self-Government of 2009 gives Greenland law-making and executive power in the fields it has taken

over, and still lists the remaining parts of the health field as ones it may take over from Denmark [33][34][35]. Kosovo's health system rests on Law No. 04/L-125 on Health, passed by its Assembly, in force since May 2013 and amended in 2022 and 2023 [36]. Every added country went through the same three steps: research in the country's own languages with primary sources first, an adversarial cross-check of the file by a second agent session of the same model family, and a search for published accounts (Section 5). Names follow our country files; listing a place separately reflects a separate health system and law and implies no position on its status. Regions are our own grouping for reading the tables (Turkey, Russia, Ukraine and Albania are in Europe; Israel is in the Middle East), not a political classification.

Use of AI. We report this following the 2025 position statement of Cochrane, the Campbell Collaboration, JBI and the Collaboration for Environmental Evidence, which asks authors to name the AI system, its version and the dates used, the purpose, and how its output was validated [22].

- *System and dates.* Research agents built on Anthropic's Claude models (Claude Opus 5.5, run through Claude Code), used from 30 September to 2 October 2026 for the countries. For the US state layer (Section 11), agent sessions ran on Claude Opus 5.5 (model id claude-opus-5-5) through Claude Code (command-line versions 2.1.288 and 2.1.289), from 3 October 2026, 13:46 UTC to 4 October 2026, 09:34 UTC: 289 sessions in all, every transcript kept, counting sessions that were voided or run again (104 research, 54 cross-check, 49 blind rating, 15 second-search, 40 pre-publication check, 18 ruling-drafting and patching, 3 audit-review and 6 pilot sessions). An agent directed by the authors drafted the version 1.1 text on 4 October 2026.
- *What the agents did.* Research and scoring of every country against the written rubric; a cross-check of each country's research by a second agent session, which reclassified every cited publisher, rechecked every link and compared each score with the anchor band for its legal situation; the consistency audit (Section 7); collection and first review of the accounts (Section 5); the outside research for the context layer (Section 9); the reading of the X sample (Section 12); the blind re-scoring below; and drafting of this paper. For the state layer: the research of each state and of Washington, DC; the cross-check; the blind re-score and the open searches; the drafting of editor rulings; the turning of each ruling into exact edits; and the pre-publication checks.
- *What the authors did.* The authors directed the research, ruled on every audit finding and every proposed score change, and reviewed

and revised the text. Each ruling is recorded with its evidence in docs/SCORE_CHANGES.md. For the state layer, the authors approved the delegated editor rulings and themselves made every ruling that moved a level or value (Section 11.7): J. A. Snyder decided, in consultation with R. P. Hill IV and D. Raney; the rulings are logged in analysis/us/states/main/rulings_applied.json. We do not claim that a person read every cell and every source.

- *How the output was checked.* A link check of every cited URL (the figures in the abstract). A verifier for the accounts, tested by planting eight rule violations, all caught. A verifier for the stability analysis, tested with planted bugs (wrong rounding, swapped weights, a score off by one). A verifier of the blind re-scoring transcripts, tested with 13 planted breaches of the blind, all caught. The cross-check of each country's research by a second agent session of the same model family, which could see the first session's scores. A blind re-scoring of a random sample of cells, reported below. For the state layer: a check of every quote against a stored copy of its page, the planted-error test of the cross-checker, a script audit of every session's transcript, a verifier of every state file, and the locked blind re-score (Sections 11.3 and 11.4). The checking agents run on the same model family as the research agents, so these checks are independent readings, not independent judges; Sections 7.4 and 11.8 set out what that limits.

The rubric requires that every number and law name come from a page the agent opened, and that a fact which could not be verified be marked "not verified" in the text. Each country file's asOf is the date the file was last checked; dates run from 2026-10-01 to 2026-10-02.

Evidence. Each category cites one to four sources, most of them dated, and each source carries its publisher class (law text, government or regulator, intergovernmental body, academic, news, law firm, blog or vendor). Each country also lists its key laws, a care-journey map (each of seven stages marked connected, partial, siloed or unknown), and recent news items.

Confidence. Each label is computed from the country file by a script (scripts/confidence.py), never set by hand, and the script that fills this paper refuses a label the rule does not give. A category is weak when it cites no primary source (law text, government, regulator or intergovernmental body) or when its text admits that a fact was not verified, matched on a fixed set of wordings ("not verified", "unverified", "could not verify", "could not confirm", "not confirmed", "did not verify", "no verified" and similar). A country is high confidence when 7 or 8 of its categories cite a primary source and none admits an unverified fact, low when 4 or more of its 8

categories are weak, and medium otherwise. The low tier was defined on 2 October 2026, before any label was recomputed. Until then the labels were stored fields written by agents: a reviewer found that many “high” countries carried the wording the printed rule says caps a label at medium, and that the source count alone no longer separated countries (177 countries cite a primary source in all eight categories). Labels now: 27 high, 105 medium and 66 low; before: 70 high, 128 medium and 0 low. Only 27 of the 70 countries then labeled high keep the label. 171 countries have at least one category whose text admits an unverified fact. The publisher classes behind the count were assigned by agents and domain rules, and the cross-check reclassified every cited publisher. The label measures how much of a file rests on official sources and on facts the research says it verified; it does not measure how much is known about practice, and a law text can make a category primary where nothing is known about how it works. 66 countries are low confidence: Albania, Algeria, Antigua and Barbuda, Argentina, Azerbaijan, Bahamas, Bahrain, Bangladesh, Barbados, Bosnia and Herzegovina, Bulgaria, Cambodia, Cape Verde, Croatia, Cuba, El Salvador, Equatorial Guinea, Eritrea, Ethiopia, Fiji, Greece, Greenland, Grenada, Guatemala, Guinea, Guinea-Bissau, Haiti, India, Iran, Iraq, Kazakhstan, Kenya, Kiribati, Kosovo, Kuwait, Marshall Islands, Mauritania, Micronesia, Moldova, Monaco, Mongolia, Montenegro, Morocco, Nauru, Nepal, Oman, Palau, Palestine, Peru, Qatar, Romania, Russia, San Marino, Sao Tome and Principe, Saudi Arabia, Serbia, Sierra Leone, Slovenia, Sri Lanka, Syria, Tonga, Turkey, United Arab Emirates, Uzbekistan, Vanuatu, Zambia.

Same regime, same band. Rubric v1.1 adds sub-anchors so that the same legal situation lands in the same band wherever it is. For example, a statutory right to a copy with no national portal starts at 45 on access, gains 5 for a free right with a statutory deadline or a structured format, and loses 5 each for a fee, no deadline, a limited scope or a right not yet enforceable. In EU and EEA states the GDPR’s one-month deadline (extendable by two further months) and free first copy bind by law, so that step is credited whatever the cited national page says. Export in a structured format is not an EU-wide floor in the same way: the GDPR’s portability right applies only where processing rests on consent or a contract, and does not apply to processing for a task carried out in the public interest (Article 20(1) and (3)) [42], which covers most public health records. A structured download becomes an EU-wide right only when EHDS Article 3(2) applies, from 2029 [5]. An EU country with only EU law on clinical AI scores 50, plus 4 for each verified national addition. Research use without consent or opt-out starts at 40 and gains 3 for each

documented safeguard, up to 50. If a key fact could not be verified, the score stays in the lower half of its band and earns nothing from that fact.

Precision. 871 of 1584 category scores (55%) are multiples of five: scores cluster there, which shows how numbers were chosen rather than how precise they are. The blind re-scoring below measures precision directly. We use competition ranking: tied countries share a rank, shown as "4=", and ties are taken on the displayed whole number.

How far a second reading agrees. To test whether the scores depend on who does the scoring, we drew a stratified random sample of 64 of the 520 country-category cells then in the index (65 countries) (8 per category, regions in proportion to country count, fixed seed). A fresh agent session re-scored each one blind. It saw only the rubric, the anchor rules with every country-specific score removed, the country, the category and the cell's cited sources. The analysis plan was written and hashed before any cell was scored. The rater ran on 2026-10-02 (model claude-opus-5-5) and is compared with the scores as published at the time of the run. On the 58 cells it could score, it landed on average 5.8 points (95% CI 4.3 to 7.6) from the published score, within 10 points in 88% of cells, with no overall bias (-0.4 points). The intraclass correlation (ICC(2,1), absolute agreement) was 0.82 (95% CI 0.72 to 0.89) and the weighted kappa on the five bands 0.63. Two agent readings of the same sources agree on ordering and level; this does not show that a human, a different model, or a rater gathering its own evidence would agree. Against the current published scores the same ratings give an intraclass correlation of 0.83 (95% CI 0.70 to 0.92; 25 of the 58 cells were re-researched after the run, and the rater read the earlier sources). A single category score does not replicate to the point: the limits of agreement are -17.7 to +16.9 points, and 38% of cells changed band. 6 cells could not be scored because the rater could not read their sources, and 43 of 168 (26%) of the cited URLs in the sample were unreadable to it, mostly PDFs; agreement was closer where every source was read. The rater shares a model family with the research agents, so this is agreement between two agent readings of the same evidence under the same rubric. It is not a human inter-rater study, and it does not test whether a rater who gathered its own evidence would agree. Agreement differs by category (exploratory, not in the plan; 5 to 8 cells each): access 0.25, research 0.54, clinical AI 0.68, clinician access 0.73, journey 0.88, commercial 0.92, privacy 0.92 and control 0.95. Access, which carries 20% of the weight and is the core of the question, agrees least: 0.25 (n 8). 5 cells differed by 15 points or more and were

returned to the authors; the rulings so far are in docs/SCORE_CHANGES.md, and one (India, clinician access) awaits a ruling.

A second full reading of 43 countries. On 2 October 2026 the first 43 countries were researched again from scratch, which works as an unplanned test-retest of the whole pipeline, research included. Between the two readings 278 of 344 category scores in 43 countries changed. Agreement on single category scores (intraclass correlation ICC(2,1), absolute agreement [32]) was 0.86 (95% CI 0.83 to 0.89), with limits of agreement of -12.4 to +15.2 points and a band change in 80 of 344 cells. Agreement on the overall score was 0.94 (95% CI 0.90 to 0.96), Spearman 0.95; the largest move was 11 points, and 11 of 43 countries changed band. Agreement was 0.86 or higher in six categories but weak in research consent (0.54) and clinical AI (0.58), which together carry 10% of the weight. Two caveats make these figures an upper bound: the second reading could see the first reading's scores, so it may have anchored on them, and both readings came from the same model family. Nor was it a pure re-reading: it added primary sources and moved research cells onto the 45 to 50 anchor (docs/SCORE_CHANGES.md), and it scored higher on average, by +1.4 points per cell (clinical AI +4.9 and research +2.7), where the blind study found no overall bias. The test-retest also showed that part of the scoring error is shared across a country, which the stability analysis in Section 4.2 now models.

A blind re-score of the added countries. Because the first blind re-scoring drew only on the first 65 countries, we ran a second, with its own plan written, hashed and committed before any of its cells was scored (analysis/reliability2/PLAN.md): a region-stratified random sample of 136 of the 1,064 cells of the 133 countries added on 2 October 2026, with more cells for access. Fresh agent sessions scored them under the first study's protocol, seeing only the rubric, the anchor rules with country names and scores removed, the country, the category and the cell's cited links, as in the first study, and opening nothing else. On the 108 cells that could be scored, the rater landed on average 5.4 points (95% CI 4.4 to 6.5) from the published score, within 10 points in 87% of cells, with no overall bias (-1.0 points, 95% CI -2.5 to 0.4) and an intraclass correlation of 0.84 (95% CI 0.77 to 0.89), +0.02 (95% CI of the difference -0.10 to +0.16) against the first study, so under the pre-registered rule the added countries keep the same error term (cell error 5.4 points against 6.2 in the first study). The limits of agreement for one cell are -15.9 to +13.8 points. Access agreed least: intraclass correlation 0.68 (95% CI 0.29 to 0.87, n 21), with the rater 5.9 points lower on average (95% CI -9.7 to -1.9), mostly where it could not open the cited law text, often a scanned PDF, and fell back on the rule that an unverified right scores

low. A single access score for an added country should not be read as reproducible to the point. The rater also spread its scores wider than ours (standard deviation 14.7 against 12.0; the slope of the difference on the pair mean is +0.21, $p < 0.001$): it went lower on low cells and higher on high ones, which the first study did not show, so our scores for the added countries sit closer to the middle of the scale than a blind reading of the same sources puts them. Of the 28 cells not scored, 20 had sources the rater could not read (169 of 422 (40%) of the cited URLs in the sample were unreadable to it) and 8 were lost when a rater session was excluded under the plan's transcript audit; all 5 excluded sessions were false alarms of the audit script, not breaches of the blind, and no session opened our site. 12 cells differed by 15 points or more and are with the authors for a ruling.

What the studies cover. The first blind study and the test-retest cover only countries among the first 65. The first blind sample was drawn from the 65 countries (520 cells) then in the index, and leans to Europe: 34 of the 58 rated cells are European and 4 African. The test-retest covers the first 43. The second blind study covers the 133 countries added on 2 October 2026, 67% of the index, so every country now sits in the frame of one blind study. No test-retest covers the added countries, so the part of the error model in Section 4.2 that is shared across a country is carried over to them from the first 43.

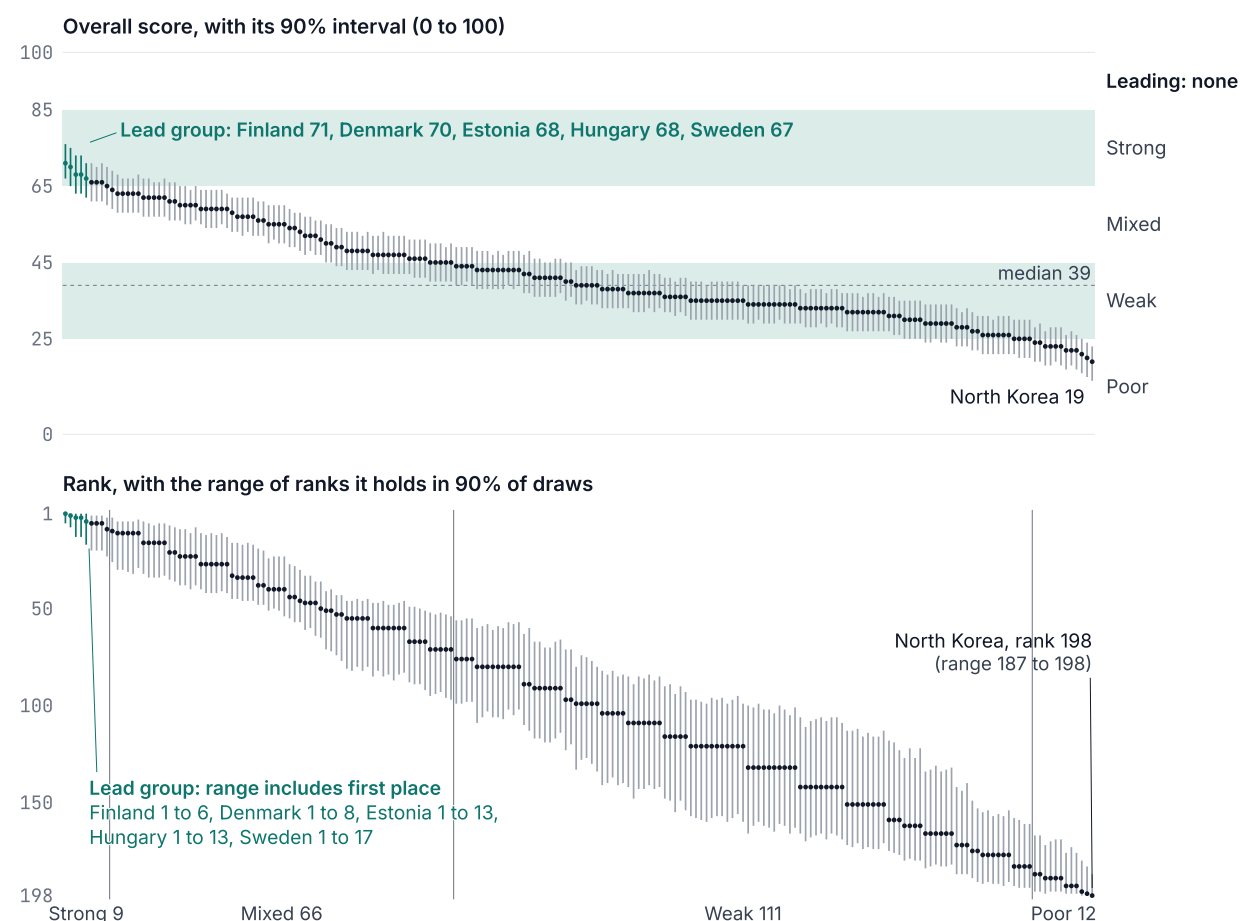
Computation. The overall score is computed from the category scores and the published weights, in the page itself and in the build script that produced the tables in this paper. No overall score is stored or typed by hand.

4. Results

4.1 Bands first, then ranks

By band: 9 Strong (7 to 14 across draws), 66 Mixed (58 to 72), 111 Weak (101 to 115) and 12 Poor (10 to 19). The ranges come from the stability analysis in Section 4.2. No country reaches the Leading band. Finland (71), Denmark (70), Estonia (68), Hungary (68) and Sweden (67) have the highest scores; allowing for scoring error, each of them is first, alone or tied, in at least 5% of draws; North Korea (19) is last. The median is 39.

Table 2 gives each country's rank with the range of ranks it holds in 90% of draws. Read the range, not the single rank. Two countries whose ranges overlap should not be called different.



Countries in rank order, left to right; every country is in Table 2. Median width of a rank range: 43 places.

198 countries, data as of 2026-10-02. Intervals: 10,000 draws of the main model (weights and scoring error varied together), analysis/robustness. Source: Health Record Rights Index, SuperTruth.

Figure 3. Every country in rank order. The lower panel draws Table 2's rank ranges; the upper panel adds the score intervals behind them. Only the lead group and the last-ranked country are labeled; look up any other country in Table 2. Bands are the labeled zones, and no country reaches Leading.

Table 2. Overall scores, ranks (competition ranking on the displayed score; "=" marks a tie), the 90% rank range from the stability analysis (Section 4.2), region, keys class, band and confidence. Data as of the date on the title page.

Rank	Rank range (90%)		Country	Region	Keys	Overall	Band	Confidence
1	1 to	6	Finland	Europe	Shared	71	Strong	high
2	1 to	8	Denmark	Europe	Shared	70	Strong	medium
3=	1 to	13	Estonia	Europe	Shared	68	Strong	medium
3=	1 to	13	Hungary	Europe	Shared	68	Strong	high
5	1 to	17	Sweden	Europe	Shared	67	Strong	high
6=	2 to	20	Australia	Oceania	Shared	66	Strong	medium
6=	2 to	20	France	Europe	Shared	66	Strong	high
6=	2 to	20	Norway	Europe	Shared	66	Strong	high
9	3 to	23	Austria	Europe	Shared	65	Strong	high
10	3 to	26	Portugal	Europe	Shared	64	Mixed	high
11=	5 to	30	Belgium	Europe	Shared	63	Mixed	medium
11=	5 to	30	Germany	Europe	Shared	63	Mixed	high

Rank	Rank range (90%)		Country	Region	Keys	Overall	Band	Confidence
11=	5 to	31	Italy	Europe	Shared	63	Mixed	medium
11=	5 to	32	Taiwan	Asia	Shared	63	Mixed	medium
11=	4 to	29	Turkey	Europe	Shared	63	Mixed	low
16=	5 to	32	Iceland	Europe	Shared	62	Mixed	medium
16=	6 to	34	Israel	Middle East	Shared	62	Mixed	medium
16=	7 to	34	Latvia	Europe	Shared	62	Mixed	medium
16=	6 to	34	Singapore	Asia	Shared	62	Mixed	medium
16=	6 to	33	Slovenia	Europe	Shared	62	Mixed	low
21=	7 to	35	Liechtenstein	Europe	Shared	61	Mixed	medium
21=	8 to	36	Spain	Europe	Shared	61	Mixed	high
23=	9 to	38	Croatia	Europe	Shared	60	Mixed	low
23=	9 to	38	Lithuania	Europe	Shared	60	Mixed	medium
23=	11 to	40	Netherlands	Europe	Shared	60	Mixed	medium
23=	8 to	38	South Korea	Asia	Shared	60	Mixed	medium
27=	11 to	40	Bulgaria	Europe	Shared	59	Mixed	low
27=	12 to	42	Luxembourg	Europe	Shared	59	Mixed	medium
27=	11 to	42	Malta	Europe	Shared	59	Mixed	medium
27=	12 to	42	Poland	Europe	Shared	59	Mixed	medium
27=	11 to	41	United Kingdom	Europe	Shared	59	Mixed	high
27=	13 to	42	Uruguay	Americas	Shared	59	Mixed	medium
33	16 to	45	Slovakia	Europe	Shared	58	Mixed	medium
34=	16 to	45	Andorra	Europe	Shared	57	Mixed	medium
34=	17 to	46	Armenia	Europe	Shared	57	Mixed	medium
34=	16 to	46	Czechia	Europe	Shared	57	Mixed	medium
34=	17 to	46	Greece	Europe	State	57	Mixed	low
38=	19 to	48	Japan	Asia	Shared	56	Mixed	high
38=	20 to	48	Romania	Europe	Shared	56	Mixed	low
40=	24 to	51	Cyprus	Europe	State	55	Mixed	medium
40=	23 to	50	San Marino	Europe	Shared	55	Mixed	low
40=	23 to	51	Serbia	Europe	Shared	55	Mixed	low
40=	23 to	54	United Arab Emirates	Middle East	Shared	55	Mixed	low
44=	27 to	55	Costa Rica	Americas	State	54	Mixed	medium
44=	28 to	57	Switzerland	Europe	Shared	54	Mixed	medium
46	30 to	57	Saudi Arabia	Middle East	State	53	Mixed	low
47=	33 to	61	Brazil	Americas	State	52	Mixed	medium
47=	33 to	60	Canada	Americas	Shared	52	Mixed	high
47=	36 to	64	Thailand	Asia	Shared	52	Mixed	medium
50	36 to	65	Georgia	Europe	Shared	51	Mixed	medium
51=	41 to	71	Mongolia	Asia	Shared	50	Mixed	low
51=	40 to	72	Qatar	Middle East	Institutional	50	Mixed	low
53=	43 to	74	Kenya	Africa	Shared	49	Mixed	low
53=	43 to	77	Ukraine	Europe	Shared	49	Mixed	medium
55=	46 to	81	Argentina	Americas	Shared	48	Mixed	low
55=	46 to	82	Azerbaijan	Asia	State	48	Mixed	low
55=	45 to	80	Kazakhstan	Asia	State	48	Mixed	low

Rank	Rank range (90%)		Country	Region	Keys	Overall	Band	Confidence
55=	46 to	82	New Zealand	Oceania	Shared	48	Mixed	high
55=	45 to	80	United States	Americas	Institutional	48	Mixed	high
60=	47 to	85	Bahrain	Middle East	Institutional	47	Mixed	low
60=	47 to	82	El Salvador	Americas	State	47	Mixed	low
60=	46 to	85	Greenland	Europe	State	47	Mixed	low
60=	48 to	84	Indonesia	Asia	State	47	Mixed	medium
60=	48 to	87	Ireland	Europe	State	47	Mixed	high
60=	47 to	84	Kyrgyzstan	Asia	State	47	Mixed	medium
60=	46 to	83	Vietnam	Asia	State	47	Mixed	high
67=	49 to	88	Belarus	Europe	State	46	Mixed	high
67=	49 to	89	Oman	Middle East	State	46	Mixed	low
67=	50 to	89	Tonga	Oceania	State	46	Mixed	low
67=	51 to	92	Uzbekistan	Asia	State	46	Mixed	low
71=	52 to	95	India	Asia	Shared	45	Mixed	low
71=	53 to	94	Kuwait	Middle East	State	45	Mixed	low
71=	52 to	93	North Macedonia	Europe	State	45	Mixed	medium
71=	53 to	96	Peru	Americas	Institutional	45	Mixed	low
71=	54 to	97	Russia	Europe	State	45	Mixed	low
76=	56 to	99	Bosnia and Herzegovina	Europe	Institutional	44	Weak	low
76=	56 to	99	Montenegro	Europe	Institutional	44	Weak	low
76=	55 to	98	Panama	Americas	Institutional	44	Weak	medium
76=	55 to	98	Rwanda	Africa	State	44	Weak	medium
80=	60 to	109	Brunei	Asia	State	43	Weak	high
80=	59 to	106	China	Asia	State	43	Weak	medium
80=	58 to	103	Colombia	Americas	State	43	Weak	medium
80=	61 to	106	Ecuador	Americas	State	43	Weak	medium
80=	59 to	106	Eswatini	Africa	Institutional	43	Weak	medium
80=	60 to	107	Malaysia	Asia	Institutional	43	Weak	medium
80=	57 to	102	Monaco	Europe	Institutional	43	Weak	low
80=	58 to	105	Seychelles	Africa	State	43	Weak	medium
80=	57 to	102	South Africa	Africa	Institutional	43	Weak	medium
89=	63 to	110	Mexico	Americas	Institutional	42	Weak	high
89=	60 to	113	Nauru	Oceania	Institutional	42	Weak	low
91=	67 to	117	Albania	Europe	Institutional	41	Weak	low
91=	67 to	117	Jordan	Middle East	State	41	Weak	medium
91=	70 to	119	Mauritius	Africa	State	41	Weak	medium
91=	70 to	119	Moldova	Europe	Institutional	41	Weak	low
91=	67 to	117	Tanzania	Africa	State	41	Weak	medium
91=	66 to	114	Zambia	Africa	State	41	Weak	low
97=	71 to	123	Bhutan	Asia	State	40	Weak	medium
97=	71 to	120	Malawi	Africa	Institutional	40	Weak	high
99=	80 to	133	Bahamas	Americas	Institutional	39	Weak	low
99=	74 to	128	Botswana	Africa	Institutional	39	Weak	medium
99=	75 to	127	Chile	Americas	Institutional	39	Weak	medium
99=	77 to	130	Philippines	Asia	Institutional	39	Weak	medium

Rank	Rank range (90%)		Country	Region	Keys	Overall	Band	Confidence
99=	80 to	134	Uganda	Africa	State	39	Weak	medium
104=	82 to	141	Bangladesh	Asia	State	38	Weak	low
104=	84 to	140	Côte d'Ivoire	Africa	State	38	Weak	medium
104=	82 to	137	Ghana	Africa	State	38	Weak	medium
104=	85 to	141	Maldives	Asia	Institutional	38	Weak	high
104=	84 to	140	Zimbabwe	Africa	State	38	Weak	medium
109=	85 to	142	Barbados	Americas	Institutional	37	Weak	low
109=	89 to	148	Benin	Africa	Institutional	37	Weak	medium
109=	85 to	143	Ethiopia	Africa	State	37	Weak	low
109=	86 to	143	Jamaica	Americas	Institutional	37	Weak	medium
109=	84 to	141	Kosovo	Europe	Institutional	37	Weak	low
109=	87 to	143	Tunisia	Africa	Institutional	37	Weak	medium
109=	85 to	142	Vatican City	Europe	Institutional	37	Weak	medium
116=	90 to	148	Cape Verde	Africa	Institutional	36	Weak	low
116=	89 to	149	Gabon	Africa	Institutional	36	Weak	medium
116=	94 to	155	Mali	Africa	Institutional	36	Weak	medium
116=	90 to	149	Morocco	Africa	Institutional	36	Weak	low
116=	93 to	154	The Gambia	Africa	Institutional	36	Weak	medium
121=	97 to	157	Algeria	Africa	State	35	Weak	low
121=	97 to	158	Angola	Africa	State	35	Weak	medium
121=	98 to	159	Burkina Faso	Africa	Institutional	35	Weak	medium
121=	99 to	160	Burundi	Africa	State	35	Weak	medium
121=	96 to	158	Cuba	Americas	State	35	Weak	low
121=	97 to	157	Djibouti	Africa	State	35	Weak	medium
121=	96 to	157	Lesotho	Africa	Institutional	35	Weak	medium
121=	99 to	160	Mauritania	Africa	Institutional	35	Weak	low
121=	97 to	158	Niger	Africa	Institutional	35	Weak	medium
121=	95 to	156	Nigeria	Africa	Institutional	35	Weak	high
121=	99 to	160	Turkmenistan	Asia	State	35	Weak	medium
132=	100 to	166	Belize	Americas	State	34	Weak	medium
132=	101 to	162	Democratic Republic of the Congo	Africa	Institutional	34	Weak	medium
132=	99 to	160	Dominican Republic	Americas	Institutional	34	Weak	medium
132=	102 to	162	Guinea	Africa	Institutional	34	Weak	low
132=	102 to	163	Lebanon	Middle East	Institutional	34	Weak	medium
132=	104 to	165	Madagascar	Africa	Institutional	34	Weak	high
132=	100 to	161	Senegal	Africa	Institutional	34	Weak	medium
132=	102 to	162	Somalia	Africa	Institutional	34	Weak	medium
132=	105 to	166	Togo	Africa	Institutional	34	Weak	medium
132=	101 to	163	Vanuatu	Oceania	Institutional	34	Weak	low
142=	106 to	166	Antigua and Barbuda	Americas	Institutional	33	Weak	low
142=	111 to	170	Central African Republic	Africa	Institutional	33	Weak	medium
142=	108 to	168	Chad	Africa	Institutional	33	Weak	medium
142=	107 to	167	Egypt	Middle East	State	33	Weak	medium
142=	106 to	166	Mozambique	Africa	State	33	Weak	medium

Rank	Rank range (90%)		Country	Region	Keys	Overall	Band	Confidence
142=	108 to	169	Palau	Oceania	State	33	Weak	low
142=	106 to	166	Paraguay	Americas	Institutional	33	Weak	medium
142=	106 to	167	Republic of the Congo	Africa	Institutional	33	Weak	high
142=	109 to	169	Suriname	Americas	Institutional	33	Weak	medium
151=	113 to	172	Comoros	Africa	Institutional	32	Weak	medium
151=	113 to	173	Equatorial Guinea	Africa	State	32	Weak	low
151=	112 to	171	Guatemala	Americas	Institutional	32	Weak	low
151=	116 to	174	Nicaragua	Americas	State	32	Weak	medium
151=	114 to	173	Saint Lucia	Americas	Institutional	32	Weak	medium
151=	116 to	174	Sao Tome and Principe	Africa	Institutional	32	Weak	low
151=	116 to	174	Sri Lanka	Asia	Institutional	32	Weak	low
151=	113 to	171	Tajikistan	Asia	State	32	Weak	medium
159=	118 to	175	Cameroon	Africa	Institutional	31	Weak	medium
159=	119 to	176	Nepal	Asia	Institutional	31	Weak	low
159=	123 to	177	Syria	Middle East	State	31	Weak	low
162=	127 to	180	Honduras	Americas	Institutional	30	Weak	medium
162=	122 to	179	Iran	Middle East	State	30	Weak	low
162=	127 to	180	Saint Kitts and Nevis	Americas	Institutional	30	Weak	medium
162=	122 to	180	Samoa	Oceania	State	30	Weak	high
166=	134 to	184	Bolivia	Americas	Institutional	29	Weak	medium
166=	133 to	183	Fiji	Oceania	Institutional	29	Weak	low
166=	131 to	182	Guyana	Americas	State	29	Weak	medium
166=	134 to	183	Kiribati	Oceania	Institutional	29	Weak	low
166=	133 to	183	Laos	Asia	State	29	Weak	medium
166=	132 to	182	Trinidad and Tobago	Americas	Institutional	29	Weak	medium
172=	142 to	187	Palestine	Middle East	State	28	Weak	low
172=	144 to	187	Timor-Leste	Asia	State	28	Weak	medium
172=	140 to	187	Venezuela	Americas	State	28	Weak	medium
175=	149 to	189	Iraq	Middle East	State	27	Weak	low
175=	153 to	191	Sierra Leone	Africa	Institutional	27	Weak	low
177=	155 to	192	Grenada	Americas	Institutional	26	Weak	low
177=	154 to	191	Liberia	Africa	Institutional	26	Weak	medium
177=	157 to	192	Namibia	Africa	Institutional	26	Weak	medium
177=	156 to	192	Pakistan	Asia	Institutional	26	Weak	medium
177=	157 to	193	Papua New Guinea	Oceania	Institutional	26	Weak	medium
177=	154 to	191	Saint Vincent and the Grenadines	Americas	Institutional	26	Weak	medium
183=	158 to	193	Cambodia	Asia	Institutional	25	Weak	low
183=	159 to	194	Micronesia	Oceania	State	25	Weak	low
183=	161 to	194	Solomon Islands	Oceania	Institutional	25	Weak	medium
183=	161 to	194	South Sudan	Africa	Institutional	25	Weak	medium
187=	167 to	196	Dominica	Americas	Institutional	24	Poor	high
187=	167 to	196	Haiti	Americas	Institutional	24	Poor	low
189=	172 to	197	Libya	Africa	Institutional	23	Poor	high
189=	170 to	196	Marshall Islands	Oceania	State	23	Poor	low

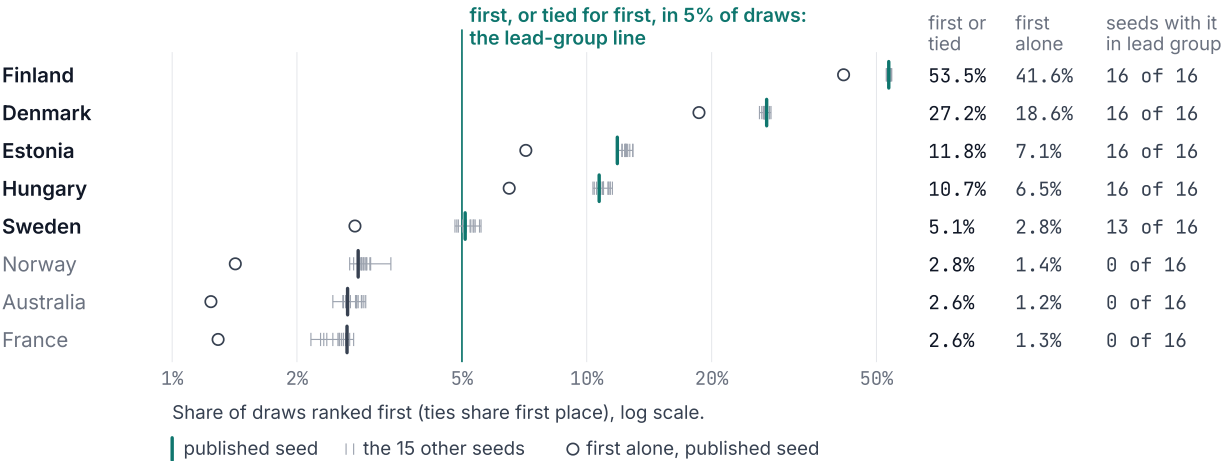
Rank	Rank range (90%)			Country	Region	Keys	Overall	Band	Confidence
189=	169	to	196	Tuvalu	Oceania	State	23	Poor	medium
189=	169	to	196	Yemen	Middle East	Institutional	23	Poor	medium
193=	176	to	197	Eritrea	Africa	State	22	Poor	low
193=	173	to	197	Myanmar	Asia	State	22	Poor	medium
193=	176	to	197	Sudan	Africa	Institutional	22	Poor	medium
196	180	to	198	Guinea-Bissau	Africa	Institutional	21	Poor	low
197	183	to	198	Afghanistan	Asia	State	20	Poor	medium
198	187	to	198	North Korea	Asia	State	19	Poor	medium

4.2 How stable the ranking is

We tested how much the ranking depends on our own choices, following the uncertainty and sensitivity analysis in the OECD and European Commission Joint Research Centre Handbook on Constructing Composite Indicators [21]. We redrew the eight weights 10,000 times around the published ones, in three ways (a Dirichlet draw, and each weight moved by up to 25% or up to 50% and then rescaled). We gave every category score a random error, first uniform up to five points, then calibrated to measured disagreement: a normal error with standard deviation 6.24 points from the blind re-scoring's limits of agreement, and a model with a standard deviation of 5 points per category plus 2 points shared across a country, the split the test-retest suggested. The main model, from which Table 2's ranges come, combines that shared error with varied weights. Separately, we rebuilt the index with equal weights, with a geometric mean, and eight times with one category left out.

Finland is first in every rebuilt index. Under the main model, the countries whose 90% rank range includes first place, with the share of draws in which each is first, are Finland (53.50%), Denmark (27.15%), Estonia (11.85%), Hungary (10.71%) and Sweden (5.09%), so no country is called first alone (our rule requires 95%). Sweden is the closest call: it is first in 509 of 10,000 draws (5.09%), just over the 5% that puts first place inside its 90% rank range. Rerun with 15 other seeds fixed in advance, its share runs from 4.81% to 5.56% (mean 5.17%), and it would fall out of the group under 3 of them. Finland, Denmark, Estonia and Hungary stay in under every seed, and no other country joins under any (the nearest, Norway, reaches 3.37% at most). The rule was set before these results and the published seed decides, so the group stands as published. With 10,000 draws the simulation's own standard error near 5% is 0.22 percentage points, so Sweden's place in the group is within simulation noise and is reported as such. First-place shares count a draw in which countries tie for first on the displayed whole number as a first for each of them, as the published ranking does, so the 5

shares sum to 108%. Counting only draws in which a country is first alone: Finland 41.64%, Denmark 18.65%, Estonia 7.13%, Hungary 6.50% and Sweden 2.76%. Sweden is the most fragile member of the group. It is first alone in 2.76% of draws, and under the error model calibrated on the blind re-scoring alone (6.24 points per category, published weights) it is first in 3.88% of draws counting ties, below the 5% cut, so under that model it would leave the group; that model's group is Finland, Denmark, Estonia and Hungary. The rule was fixed before the results and counts shared first place, as the published ranking does, so the group stands as published: read it as the countries that are first, alone or tied, in at least 5% of draws under the main model. No country reaches the top band in any draw; 12 countries are Poor as published (10 to 19 across draws): the highest score seen is 83 and the lowest 8. The median country stays Weak (39 to 41). The rebuilt rankings correlate with ours at 0.99 or above (Spearman). Under the main model half the countries have a 90% rank range of 43 places or fewer; under the cell-only calibration the median range is 37 places, and under the uniform five-point error we first used, which is a lower bound, 17. 100 of 198 countries keep their band in 95% or more of draws.



Shown: every country in the lead group under any seed, or first in at least 2% of draws under any seed. 10,000 draws per seed of the main model; ranks are competition ranks on the rounded score. Monte Carlo standard error at 5%: 0.0022. Seeds listed in analysis/robustness/seed_sweep.json. Source: Health Record Rights Index, SuperTruth.

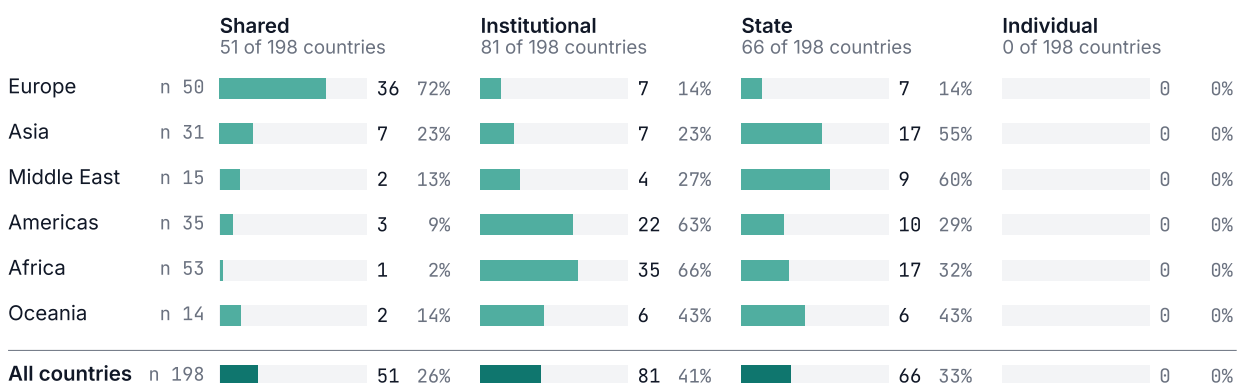
Figure 4. The lead group barely depends on the random seed. Four countries are in it under every seed; Sweden, on the 5% line, is in it under 13 of 16 seeds, and is first alone in 2.8% of draws. First counts ties: tied countries share first place.

The band counts are softer than the ranking. 84 countries keep their band in fewer than 90% of draws because they sit near a band line: 50 of them sit within two points of a band line on the unrounded score (a line falls at 24.5, 44.5, 64.5 and 84.5, where rounding changes the band), and the other 34 between 2 and 3.6 points from one. The full list, with unrounded scores, is in analysis/robustness/ROBUSTNESS.md. If a rater were generous or

harsh by up to five points across a whole country, rather than category by category, Finland would stay first in 53.7% of draws and ranks would loosen further. The test-retest in Section 3 suggests that part of the error is shared across a country, which is why the main model includes a shared offset. The code, the seed (20261002) and every setting are released with the index.

4.3 Where control sits by default

By keys class: 51 Shared, 81 Institutional and 66 State; 0 Individual. All 9 countries in the Strong band are Shared: the person has real controls (a consent register, an opt-out, an access log, a portal) inside a system run by the state or by providers. This link partly holds by construction, since a working choice over sharing also raises the control score, which carries 20% of the weight. The Individual class is empty by definition rather than by finding: it requires that nothing flow into a state or provider system by default, which no national system does.



Bar: share of the region's countries in each class, on one 0 to 100% scale (the grey track is 100%); figures: count and share. Regions in the order of Table 3. Classes as defined in Section 2.3 (rubric v1.1 anchors). Data as of 2026-10-02. Source: Health Record Rights Index, SuperTruth.

Figure 5. Who holds the keys, by region. Shared systems are mostly European, State systems lead in Asia and the Middle East, Institutional systems lead in the Americas and Africa, and no region has an Individual system. Bars show the share of each region's countries in each class, on one scale; each cell prints the count and the share.

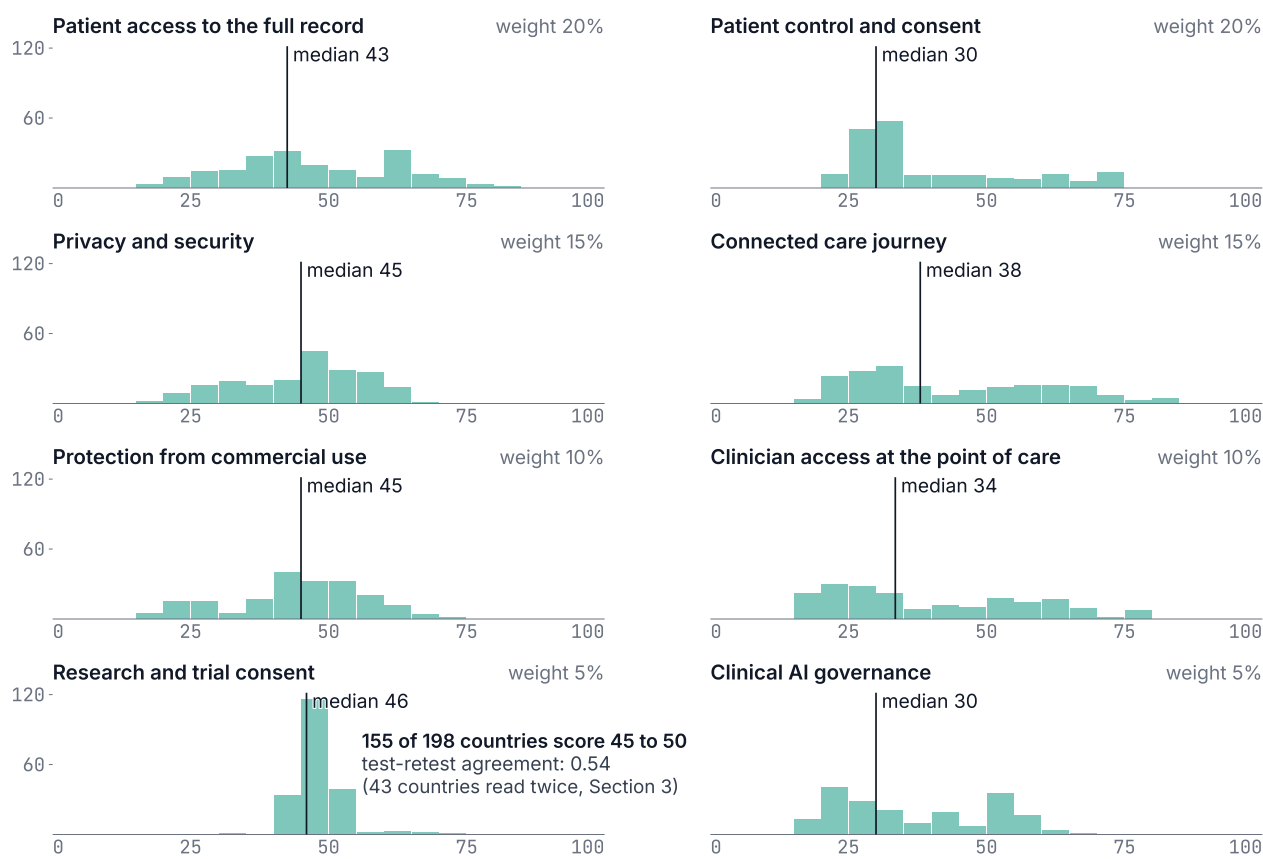
The classes describe law in force on each file's data date. EU law will move them on a legal calendar. From 2029 and 2031, depending on the data category, the European Health Data Space Regulation gives everyone in the EU a right to restrict health professionals' access to their record (Article 8) and a right to see, free, who has accessed it (Article 9) [5]. Under the Shared anchor every EU state will then qualify, and that change will come from the Regulation, not from national policy. EU law does not adopt the Individual class: Article 10 leaves an opt-out from sharing in care to each member state [5], and the adopted Regulation's opt-out rule for secondary use leaves much

to national implementation [56], after earlier warnings about the draft [55]. Whether individual control is the right frame is itself contested. The data-solidarity argument holds that governance should rest on solidarity rather than on individual control [63, 64]; “data sovereignty” carries several different meanings in the literature [65]; and collective rights over health data, such as the CARE principles for Indigenous data governance, fall outside a person-by-person class [66]. The index does not score collective rights, which matters for Greenland, New Zealand, Australia and Canada among others.

4.4 Which rights lag

Table 1 gives the spread in each category. The highest median is research and trial consent (46); the lowest is patient control and consent and clinical AI governance (30). The widest spread between the first and third quartiles is in clinician access at the point of care (31 points); the narrowest is in research and trial consent (4). A spread is not a cause, and gaps of a few points between categories are within the precision of a single score.

Research consent barely separates countries. It has a standard deviation of 4.3 points and an interquartile range of 4 points, because the anchor for research use without consent or opt-out (start at 40, plus 3 for each documented safeguard, up to 50) puts most countries in a ten-point window. By a text match, 26 research summaries say that no rule was found, and they score 40 to 50. It is also the least reliable category: its test-retest agreement is 0.54 and its blind agreement 0.54. It accounts for 0.7% of the variation in overall scores (Table 1b). We report it as scored in this version; Section 7.4 sets out the change still planned.



Countries per 5-point bin; the same axes in every panel (count axis 0 to 120). 198 countries, data as of 2026-10-02. Weights as published. Source: Health Record Rights Index, SuperTruth.

Figure 6. Which rights lag: the scores in each of the eight categories, on the same axes, with the median marked. Patient control and clinical AI governance sit lowest; research consent heaps just below the middle of the scale, which is why it barely separates countries.

4.5 By region

Table 3. Mean overall score and range by region. Regions differ in how many countries we rated; a regional mean over few countries says little.

Region	Countries	Mean overall	Range
Europe	50	56	37 to 71
Asia	31	40	19 to 63
Middle East	15	40	23 to 62
Americas	35	37	24 to 59
Africa	53	35	21 to 49
Oceania	14	34	23 to 66

4.6 Rights and delivery, separately

Table 3b splits the overall score into the two parts defined in Section 2.4, each the weighted average of its categories with the printed weights rescaled to sum to 100 within the part. Both parts track national income (GDP per head, World Bank [31]): rights at +0.67 (95% interval +0.58 to +0.75, n 188) and delivery at +0.77 (95%

interval +0.70 to +0.81, n 188). Delivery tracks income more closely, but the two intervals overlap. Across countries the two parts correlate at +0.82. So the link between scores and income is not only a matter of infrastructure: the rights sub-score tracks national income too, although it also carries some portal and register credit (Section 2.4). These sub-scores appear in this paper only.

Table 3b. Rights and delivery sub-scores across all countries, computed from the published category scores (analysis/external_corr.py), with the Spearman correlation of each with GDP per head and its 95% bootstrap interval.

Sub-score	Categories (printed weights, renormalized)	Min	Q1	Median	Q3	Max	Spearman with GDP per head
Rights	access 20, control 20, privacy 15, commercial 10, research 5	18.9	35.0	40.9	50.7	70.0	+0.67 (95% interval +0.58 to +0.75, n 188)
Delivery	journey 15, clinician access 10, clinical AI 5	16.5	25.8	35.2	53.7	76.0	+0.77 (95% interval +0.70 to +0.81, n 188)

5. What people report

5.1 Method

Everything in this index, the scores and the accounts alike, comes from public information: laws, government and regulator pages, court decisions and published news reports. We link to every source.

Alongside the scores we collected published accounts of real problems people have had with their health records: records refused, delayed or charged for; records that were wrong; breaches; records sold or shared without consent; records lost between providers. Research agents searched on 2026-10-01 to 2026-10-03 in 94 languages, local languages first, and found accounts in 50. The window was two years: material published between 1 October 2024 and 1 October 2026, and nothing older.

The rules below were written before collection by an internal privacy review (an agent working to a written brief) and approved by the authors. No outside legal review was commissioned. They are binding on every item:

- **Allowed sources:** published decisions of regulators, data protection authorities and ombudsmen; court and tribunal judgments; official parliamentary records; journalism from an edited outlet, whether or not the patient is named; case stories published by patient organizations that state the person agreed; personal blogs only with the author’s dated written consent (none were collected for this version).
- **Excluded:** every social media platform and forum; anyone under 18, or events from when they were; deceased persons unless the source is a regulator, court or parliamentary record.

- **What we publish:** our own neutral summary of no more than 40 words and a link to the original. We never name the person. We name a hospital or company only where a regulator, court or the organization itself has stated what happened. A non-English source is marked as such, with the summary ours.
- **Separation:** stories never feed a score. A release check fails if any story's link is also a source for a score.
- **Verification:** an agent reviewer opened every item. A verifier rechecks every rule flag and every link; it checks the flags the collecting agent set, and does not read each summary for names or conditions. Its run of 6 October 2026 checked all 463 accounts and found 0 of their links not answering and 0 rule failures. We tested it by planting eight rule violations, and it caught all eight. Eight of eight is a small test: it is consistent with a true catch rate well below 100%.

These are accounts we could find and publish under these rules. They are not a sample. Counts reflect where people speak publicly, where regulators publish their decisions, and where we searched; they do not measure how common problems are.

5.2 What we found

We collected 463 accounts covering 147 countries, published 2024-10-03 to 2026-10-03. Of these, 178 rest on a regulator, court or ombudsman finding, 102 on the organization's own admission, and 183 on an account not yet tested. No account names the person. The tables also suppress any count below five; no cell in Tables 4 to 6 is that small.

Table 4. Accounts by theme.

Theme	Stories
Breach	181
Record wrong	75
Sold or shared without consent	68
Access delayed or charged	40
Other	39
Access refused	34
Lost between providers	26

Table 5. Accounts by source type.

Source type	Stories
News report	346
Regulator or ombudsman decision	100
Court or tribunal judgment or parliamentary record	17

Table 6. Accounts by region. We publish counts by theme and by region as separate tables only; there is no cross-table, and the individual accounts are linked from the interactive index rather than deposited with this paper, so that an account can be taken down from the index on request. For about an hour on 2 October 2026 the accounts were public in the repository; copies made then cannot be recalled (Data and Code Availability).

Region	Stories
Europe	187
Americas	96
Asia	85
Africa	50
Middle East	26
Oceania	19

News reports are the largest source type. They count whether or not the patient is named, and we never name the person. Where a country shows few or no accounts, that reflects what is published and findable, not an absence of problems.

6. Related work, and how our scores compare with other indices

6.1 Related work

We found no index whose object is a person’s rights over their own health record. The nearest is the Euro Health Consumer Index, which in its 2018 edition scored 35 European countries on 46 health-system indicators, one of them “Access to own medical record” [19]. The same organization’s 2009 report on patient empowerment scored 31 European countries on a set of patient-rights and information indicators that included whether patients can read their own medical records [20]. Both are gray literature from a private company, and in both, record access is one item in a broader health-system or empowerment score.

Other work compares and describes without scoring. Essén and colleagues compared patient-accessible record policy and services in ten countries; all ten gave patients some right of access, and they differed on login security, proxy access and how soon results appear [7]. Kharko and colleagues surveyed experts in 29 countries on online record access: 23 had it, and clinical notes were available in 12 [8]. The NORDeHEALTH group surveyed 29,334 portal users in Norway, Sweden, Finland and Estonia [9], set out five principles for record access under the European Health Data Space (right of access, proxy access, patient-entered data, rectification and access control) [10], and proposed a framework for comparing countries [11].

Those five principles map closely onto our access and control categories. For Africa, Munung and colleagues tabulated data protection laws in 37 countries, including whether health data is a special category and which data-subject rights exist [12]; Townsend and colleagues found no AI-specific law in the 12 African countries they studied [13]. Alegre and colleagues mapped electronic record and telehealth laws across Latin America [14]. Essén and colleagues compared health app policy in nine countries, the only cross-country work we found that touches our commercial category [15]. The nearest official work is the OECD's. Its 2015 study set out eight key data governance mechanisms [57]; the OECD Council adopted its Recommendation on Health Data Governance on 13 December 2016, and a 2022 report documents how adhering countries implemented it from 2016 to 2021 [58]. A 2019 to 2020 OECD survey of 23 countries counted national health data infrastructure and governance practices country by country [59]. That work rests on government reports and its object is the system's governance; ours is the person's rights, scored from public sources. WHO's global strategy on digital health 2020 to 2025 [60] sets out the aims its Monitor [4] tracks. Law can also be coded and compared as data: legal epidemiology [61] and the methods literature on measuring law for evaluation [62] use written coding protocols and independent coders, the standard against which our single-agent coding and the absence of human coders should be judged (Section 7.4).

Other literature measures what record access does. Opening clinicians' notes to patients was taken up widely after early studies found that patients read them, reported benefits and few concerns [44, 45], and has since spread internationally [46]. Systematic reviews report increased convenience and satisfaction and some safety benefits, with study quality often low [47, 49], and a Cochrane review calls for consistent outcome measures and attention to the risk of widening inequalities [48]; an umbrella review found portal features reported too unevenly to compare across studies [50]. These studies measure use and effects; we score rights and infrastructure, and use only Eurostat's survey of use as a check (Section 6.2). On research use, comparative work finds that differences in data protection law between jurisdictions are a significant barrier to multisite research [74], and argues that EU law needs an appropriate research exemption from consent, with debated conditions [73]. On commercial use, few studies check whether health apps handle data as their privacy disclosures say [70]; de-identified data sales and patient governance are framed by the legal literature on medical big data [71]; and much of a person's digital health footprint lies outside health law [72], including outside HIPAA [69].

Our contribution is a scored, sourced and repeatable comparison with the patient's rights as its object.

6.2 Comparison with other indices

Several indices measure neighboring things. We compared country orderings using Spearman rank correlation with 95% percentile bootstrap intervals (4,000 resamples; resamples with no variation are dropped). This is a sanity check. It does not validate our scores. The other indices describe earlier years (2018 to 2024), mostly rely on government self-report, and measure digital maturity rather than patient rights. Table 7 shows 10 rows, one per external measure and matched category; all 15 computed rows are in the repository (analysis/external/external_corr.json).

Table 7. Rank correlations between our categories and published indices, for the countries in both. Computed 2026-10-06 on the current scores (the paper generator refuses this table if it was computed on other data). The European Commission, OECD and Bertelsmann values were transcribed for the original 43 countries only, so their rows cover those; the WHO values were matched for every country in the index that WHO publishes, through the UN M49 codes of every country except Kosovo and Taiwan, which have none (analysis/external/iso3_to_m49.json, from the UN Statistics Division table). Our data cites the European Commission indicator for three countries; without them, rho is +0.45 (n 11).

External measure (data year)	Our category	n	rho	95% interval
European Commission Digital Decade eHealth indicator (2024) [1]	Access	14	+0.47	-0.16 to +0.88
OECD EHR technical and operational readiness (2021) [2]	Journey	18	+0.46	-0.05 to +0.83
OECD EHR governance for analytics (2021) [2]	Research	18	-0.42	-0.73 to +0.00
Bertelsmann #SmartHealthSystems (2018) [3]	Overall	16	+0.14	-0.48 to +0.67
WHO Global Digital Health Monitor, overall (2023) [4]	Overall	48	+0.59	+0.37 to +0.76
WHO Global Digital Health Monitor, exchange architecture (2023) [4]	Journey	48	+0.38	+0.11 to +0.61
WHO Global Digital Health Monitor, AI protocol (2023) [4]	AI	47	+0.53	+0.27 to +0.71
WHO Global Digital Health Monitor, privacy laws (2023) [4]	Privacy	48	+0.55	+0.33 to +0.72
Eurostat, people who accessed personal health records online (2024) [30]	Access	34	+0.58	+0.24 to +0.82
GDP per head, PPP (World Bank, 2022 to 2024) [31]	Overall	188	+0.72	+0.64 to +0.79

Not every correlation is positive. Weak correlations would fit an index that measures the patient's side rather than system maturity, but they fit scoring noise equally well: a single cell's limits of agreement are about plus or minus 17 points (Section 3). Intervals that clear zero (6 of the 10 rows): WHO Global Digital Health Monitor, overall (2023); WHO Global Digital Health Monitor, exchange architecture (2023); WHO Global Digital Health Monitor, AI protocol (2023); WHO Global Digital Health Monitor, privacy laws (2023); Eurostat online record use (2024); and GDP per head. For privacy, the correlation over every country with a 2023 Monitor value, not only full responders, is +0.54 (95% interval +0.41 to +0.65, n 140). Two further comparisons say more than any of these. The first is national income. Our overall score correlates with GDP per head (World Bank [31], purchasing

power parity, 2022 to 2024) at +0.72 (95% interval +0.64 to +0.79, n 188); income alone accounts for 51% of the variance in our scores, about 7.7 points per natural-log unit of income. Within Europe the link is weaker: +0.47 (+0.16 to +0.71, n 47). On the 48 countries with a full 2023 Monitor response, the Monitor's overall phase correlates with income at +0.37 and our overall score at +0.54; with income held constant, the Monitor and our score correlate at +0.50 (95% interval +0.23 to +0.70), so the agreement survives with income held constant: it is not only shared income. The countries furthest above the score their income predicts are Finland (+18.9), Hungary (+17.7), Estonia (+17.6), Denmark (+15.5), Malawi (+15.2) and Kenya (+14.1); furthest below are Guyana (-24.8), Dominica (-20.3), Trinidad and Tobago (-18.8), Libya (-18.3), Saint Vincent and the Grenadines (-17.7) and Saint Kitts and Nevis (-17.7). The second is practice. Eurostat's 2024 household survey [30] asks people whether they accessed their personal health records online. Across the European countries in both, that share correlates with our access score at +0.58 (95% interval +0.24 to +0.82, n 34). It is self-report and covers Europe only, so it is a convergent check, not ground truth. The gaps run both ways: Germany 5.1% against an access score of 64, measured before Germany's 2025 rollout of its national record; in Albania 48.7% of people report online access, although our file finds no national portal that shows the record (the government portal shows a person's own e-prescriptions). The largest disagreements are informative. The European Commission indicator rates Norway's and Germany's online access well above our access scores (68 and 64). The OECD readiness survey rates Germany well below our journey score (66), but the survey predates Germany's 2025 rollout of its national electronic record; it rates Japan well above ours (48). In the WHO monitor, the figures labeled 2024 repeat the 2019 figures row for row. For many countries the 2023 overall score rests on only two legal questions, so we used only the 48 countries that answered the full 2023 survey.

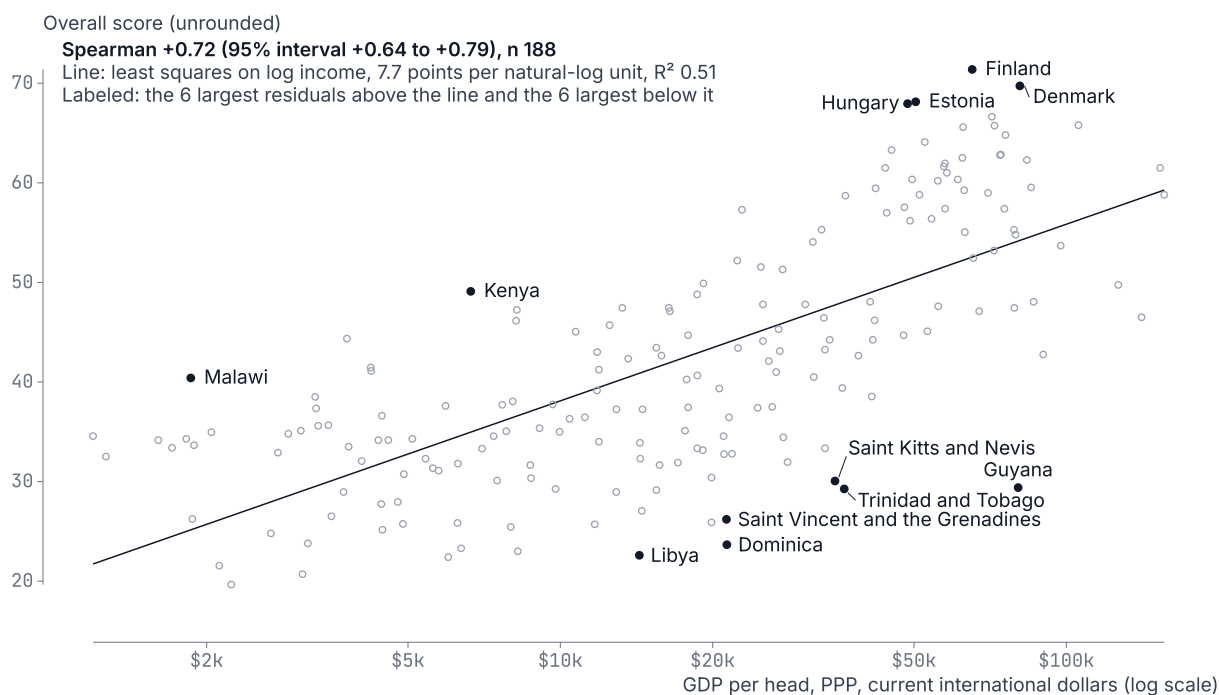


Figure 7. Richer countries score higher, but income does not decide the score. Overall score against GDP per head (World Bank [31], purchasing power parity, log scale), with the least-squares line on log income; the six largest residuals above and below the fit are named.

7. An audit of our own scores, and limitations

7.1 What the audit found (state on 1 October 2026)

Before publication an audit agent checked the original 43 countries against their own rubric. The audit covers 43 of 198 countries. The countries added later went through the cross-check in Section 3, which compared each score with the anchor band for its legal situation, but no count of the breaches it found or fixed was recorded. This section and the next describe the scores as they stood on 1 October 2026; the 2 October re-research of the same countries, described at the end of Section 7.2, re-scored them again, and Table 2 shows the result. The audit found seven problems in the scores:

- 1. The same legal situation was sometimes scored differently in different regions.** Countries with a legal right to a copy of the record but no national portal were scored anywhere from 24 to 60. Ireland's access score (24, Poor) was the clearest case: its own text describes a working legal right that is used heavily.
- 2. Identical EU clinical AI situations carried different scores,** from 50 to 62.

3. **Research use without individual consent or opt-out** was scored from 45 to 64.
4. **One country's privacy score rested on a large breach outside health.**
5. **One country ranked in the top ten on very few official sources.**
6. **Confidence labels did not track sourcing.**
7. **Some cells said a key fact was not verified but still carried a number** that read as if measured.

7.2 What we changed

The authors accepted the audit's suggested decision rules on 1 October 2026. They became the rubric v1.1 anchors (Section 3), and the original 43 were rescored against them. In all, 44 category scores changed in 33 countries, and 18 confidence labels changed. Of the score changes, 6 went up and 38 went down. The largest moves were Ireland access 24 to 45, Norway research 64 to 43, Estonia research 64 to 49, Sweden research 60 to 46, Austria clinical AI 62 to 50, South Korea privacy 78 to 66 and Spain clinical AI 62 to 50. Three countries changed band: Norway (Strong to Mixed; Strong again after the 2 October re-research), Chile (Mixed to Weak) and New Zealand (Mixed to Weak; Mixed again after the 2 October re-research); each move is within the precision set out in Section 4.2. On the 65-country table of that day, The largest fall was South Korea (7 places); the largest rise was 2 places (Spain, Latvia, Slovenia, Singapore, Iceland, United States and South Africa). Confidence became the fixed rule in Section 3.

On 2 October 2026 three internal reviews (methods, policy and law, and data integrity) and the blind re-scoring led to a second round: 6 category scores changed in 6 countries, 3 keys classes changed and 3 confidence labels changed. The GDPR deadline and free first copy are now credited by law in every EU and EEA access cell. The United States and Lithuania access cells, which sat above where their own regime's arithmetic put them, were brought onto it. A one-region portal now lifts an access cell within the "no enforceable right" band, never out of it. Every cell in the upper half of its band that admits an unverified fact was re-read; two moved. The keys anchors in Section 2.3 were written and applied, and the confidence cap was widened. The reviews were drafted by agents working for the authors; they are internal pre-deposit reviews, not journal peer review.

The same day the first 43 countries were researched again from scratch (Section 3). That re-research changed 278 of 344 category scores in 43 countries and 6 keys classes, and it superseded several of the 1 October changes above, including two of the band moves. A

second round of internal reviews, also drafted by agents, followed; its fixes to this paper are text and analysis, and the confidence labels were recomputed under the rule in Section 3.

Every change, with the evidence for it, and every cell that stayed, with the reason, is listed in the repository (docs/SCORE_CHANGES.md).

7.3 Edge cases still open

The rules settle most of the audit's findings. These cases remain, and we report them rather than force them:

- **Research cells outside the no-consent band.** Some cells whose own text describes research use without consent or opt-out sit below the band (the rule caps scores and never raises them), and one sits above it.
- **One national AI law with several health duties.** Italy's clinical AI score (54) rests on one national AI law. The anchor does not yet say whether one law with several health duties counts once.
- **Gaps in the anchors.** The anchors do not yet cover a portal with no verified legal right, a research prohibition, or binding non-EU AI rules without verified change control.
- **First-copy fees.** Some EU and EEA access cells describe copy fees without saying whether the first copy is free. Their scores sit in the portal bands, where the step does not apply, so no score moves, but the text needs a fee note.
- **United States law the cells left out (closed in version 1.1).** The access cell rests on HIPAA's right of access. It does not describe the 21st Century Cures Act information-blocking rule, under which a practice likely to interfere with access to, exchange or use of electronic health information is information blocking [75]; health IT developers and health information networks face civil money penalties of up to \$1,000,000 per violation, and providers are referred for disincentives [76], set by a 2024 rule effective 31 July 2024 [77]. The rule is why US patients now see many results and notes in their portals at once [67, 68]. The commercial cell says the FTC Health Breach Notification Rule "requires notice of breaches. It does not ban sharing." That understates it: since April 2024 a breach under the rule includes an unauthorized disclosure [78], and in its first action under the rule the FTC's proposed order barred GoodRx from sharing health data for advertising, with a \$1.5 million civil penalty [79]. We judge that neither would move a US score under the current anchors: access stays in the regime "statutory right, no national record" (50), and the FTC rule still does not require consent for every use (commercial 38). Version 1.1 added both rules to the US cell texts after a federal law audit on 3 October 2026; no US score changed.

- **The rule acts on what a cell says.** The cap on unverified facts is applied by reading each cell's text. A per-cell record of the regime each cell was placed in, and of whether a key fact is unverified, would let a reader check "same regime, same band" mechanically. It is not built in this version.

7.4 Limitations

1. **Ranks are approximate.** Section 4.2 gives each country a rank range. Read bands before ranks, and ranks as a range.
2. **A single category score carries real judgment.** The blind re-scoring puts the limits of agreement for one cell at -17.7 to +16.9 points, and the second, on the added countries, at -15.9 to +13.8. The rater was an agent session from the same model family as the research agents. Two readings from one model family may share the same blind spots, so their agreement may be higher than agreement with a human rater or a different model would be. No human inter-rater study has been done, although coding law with written protocols and independent human coders is standard practice in legal epidemiology [61, 62].
3. **The average lets strength offset weakness.** A weighted arithmetic mean lets a strong score in one category make up for a weak score in another: good infrastructure can offset weak consent. A geometric mean, which penalizes imbalance, ranks the countries almost identically (Spearman 0.99), so the choice does not drive the ranking here, but the index does not say that every right must be met.
4. **Two categories overlap.** The connected care journey and clinician access move closely together ($r = 0.97$ across 198 countries). Together they carry 25% of the weight, so connected infrastructure is in effect counted twice. Leaving either one out moves the ranking little (average rank shift 4.4 places without journey, 3.0 without clinician access, against 4.7 without control), so we kept both at full weight in this version.
5. **Breach evidence depends on reporting rules.** The privacy category counts major breaches. Breaches are only visible where law requires them to be reported and published, and the authors of two US breach studies judge that even the published US counts understate the true number, because breaches go unreported and small ones are excluded [27, 28]. A country with mandatory public reporting can look worse than one whose breaches stay unseen.
6. **No equity criterion.** A right can work on average and fail for groups: in the United States, Black and Hispanic adults were less likely than White adults to be offered a patient portal and, when offered, to access it [29]. The rubric scores national rights and infrastructure and has no criterion for who can use them. Controls can exist and be unequally understood and used: in Australia, opting out of My Health

Record was more common among the university-educated, people with health conditions and priority populations [51], and few studies of portal interventions measured disparities [53].

7. **Sources.** The index rests on 5147 cited sources (979 undated). Some laws are cited from secondary summaries rather than official text, and a few cells cite tertiary sources such as Wikipedia. Across all 7156 citations (3537 distinct pages) in category sources, laws and news, 7156 cited links checked: 7011 opened, 0 dead (HTTP 404 or 410), 88 blocked by bot protection and 57 unreachable, unresolved or answering with a server error. A link that opens shows the page exists, not that it says what the summary says; apart from the blind re-scoring, no check compares each summary with its source.
8. **Sites that block agents.** Some official sites refuse automated access. Where they did, a cell can rest on secondary sources, so confidence partly reflects whether a government's website admits agents.
9. **Time.** Scores describe the date on the title page. Several systems are changing quickly, in particular in Europe ahead of the European Health Data Space [5].
10. **Coverage.** 198 countries and territories (all 193 UN member states, plus Greenland, Kosovo, Palestine, Taiwan and Vatican City); 66 are low confidence. Places with their own health systems but no seat at the United Nations, other than those named, are not rated. The countries added on 2 October 2026 were researched in one day, many small states rest on few sources, and only the second blind re-scoring covers them, with access agreeing least; no test-retest does (Section 3).
11. **Known limits, still open in version 1.1.** These are named here with what each would change. Version 1.0 planned to address them in this version; none has been, and each will be decided, with a public changelog, in a later version.
 - *Practice credit from the operator's own account.* The anchors give "works in practice" credit from government and vendor pages. Hungary's control score (72) credits controls the person can use, while its own file says: "Only about 0.6 percent of people have changed the default." Nauru's connected care journey (65) and clinician access (68) rest on 4 sources: 1 academic coverage study, 2 pages from the system's vendor and 1 law, against New Zealand's 40 and 40, and the anchors do not say how a four-facility system compares with one of thousands. A rule requiring at least one source independent of the operator before practice credit is given would lower such cells.
 - *Research "no rule found".* Research cells in which no rule was found sit in the same 40 to 50 window as cells with a documented regime (Section 4.4). Scoring "no rule found" low, as an absent

right is scored on access, would widen the research category; at 5% of the weight, even a fall to 0 would move an overall score by at most 2.5 points.

- *A legal audit.* No lawyer has checked a cell. A stratified human legal audit of a sample of cells, in each cell's language, would give an error rate with an interval.
- *A right of reply.* No government was asked to correct facts before publication. A dated right of reply and correction window would be offered, with corrections logged in docs/SCORE_CHANGES.md.
- *The evidence grade.* Section 8 uses the tier names of the authors' own framework and puts most countries in its top two tiers; it would move to a supplement.

8. How strong is the evidence? A grade adapted from the Data Trust Index

8.1 What it is

SuperTruth's Data Trust Index (DTI) grades health data records before an AI system uses them, on eight weighted dimensions [6]. The DTI is the authors' company's own framework, and [6] is the first author's own deposit; it has not been peer reviewed. We apply its dimensions and weights to a different object: the evidence behind each score. One cell is one country and one category, with its summary, its detail and its cited sources. Each cell gets a grade from 0 to 100. A country's grade is its cells weighted by the index's category weights.

This is an adaptation. Several dimensions have no direct counterpart in published evidence:

Table 8. The DTI dimensions as adapted to the evidence behind a score.

Dimension (weight)	As adapted for evidence
Provenance (25)	How close each publisher is to the fact: law text, government or intergovernmental body highest, then academic, news, law firm, blog or vendor
Consent (20)	Open availability: whether anyone can open the source today. The furthest stretch, since published evidence has no data subject
Recency (15)	Time since publication; a law in force counts as current at any age
Quality (10)	Completeness of the evidence record, including a publication date for every source
Concordance (10)	Number of independent publishers
Validation (10)	Whether the text admits that a fact was not verified
Breadth (5)	Number of distinct kinds of evidence
Stability (5)	Whether the cell sits in an open finding of the consistency audit

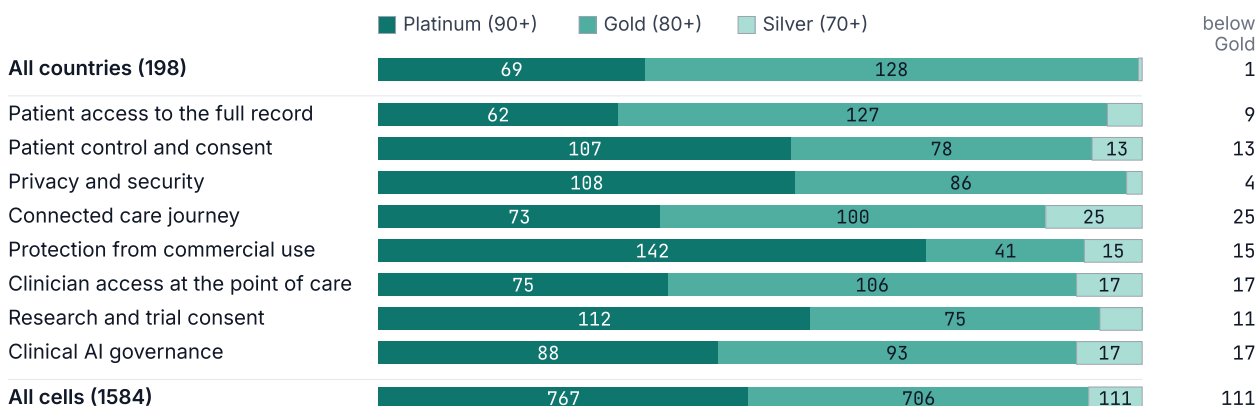
Tiers follow the DTI paper: Platinum 90 and above, Gold 80 to 89, Silver 70 to 79, Bronze 55 to 69, and below that, Below Bronze. Tiers use the unrounded grade, so a grade shown as 80 can be Silver and one shown as 90 can be Gold.

Our extension. The DTI paper defines no cap. We added one: a cell with no primary source cannot be labeled above Silver, and neither can a country with fewer than 4 of its 8 categories citing a primary source. The number is never changed, only the label. The cap applied to 18 cells and 0 countries.

8.2 Results

Country grades run from 77 to 94: 69 Platinum, 128 Gold, 1 Silver (198 countries with a published grade). No country is provisional, because one of its sources is cited from a shared file host instead of the publisher. Across all 1584 cells: 768 Platinum, 705 Gold, 111 Silver.

197 of 198 countries grade Gold or better on the evidence behind their scores.



Bars: share of cells (one country, one category) or of countries in each tier; numbers: counts. Below Silver: none. Grades are of the evidence behind a score, not of the score. DTI dimensions and weights adapted as in Section 8 (Table 8); run 2026-10-02. Source: Health Record Rights Index, SuperTruth; scripts/dti_evidence.py.

Figure 8. The evidence behind almost every country's scores grades Gold or better on the adapted Data Trust Index; the connected care journey has the most cells below Gold. Share of cells in each tier by category, with the country grades above and all cells below. The grade is of the evidence, not of the score.

8.3 What it is not

- **It does not grade the country.** A Gold grade means the evidence behind the scores is well sourced. It says nothing about whether the country protects patients well. A country can have strong evidence for a weak score.
- **It does not change any score.** No category score, overall score or rank is touched by it.
- **It is not the confidence label.** Confidence counts categories with a primary source and categories whose text admits an unverified fact; the grade also weighs whether sources open, their dates and how

many publishers agree. A low-confidence country can grade Gold. Where the two disagree, read the confidence label, which is the stricter test. The grade's cap counts primary sources only (fewer than 4 primary categories), so it is not aligned with the confidence rule.

- **It does not read the sources.** It checks who published a source, whether the link opens, when the source was published and whether the author admitted a gap. It does not check that the page says what the summary says.
- **The tier names are borrowed, not their meanings.** In the DTI paper, Gold means suitable for clinical decision support. That sentence describes health records and does not carry over to evidence.
- **It is generous.** On the published weights, a cell resting on one dated news article that opens and admits no gap reaches Gold. Two dimensions vary little across cells, which pushes most grades up.

9. Strain and split: context, not scored

9.1 The question

This layer asks a question in four links. A public system short of doctors and nurses for the demand produces long waits. People who can pay turn to private care. Public and private providers keep different record systems, so the person's record breaks apart. The layer measures each link in that chain from public data, country by country, and says where the data is silent. It changes no score, and the build fails if any score moves when it loads.

Outside work supports the first two links only in part. In Britain, buying private health insurance is associated with longer local NHS waiting lists [24]. In Australia, the expected wait does not raise the chance that a person buys insurance; a high chance of a long wait does, and on average waiting time has no significant effect [25]. The link from waits to private care is real in some systems and conditional in others.

9.2 Indicators and sources

Table 9. The four links and their sources.

Link in the chain	Indicator	Source
Too few staff	Medical doctors, and nursing and midwifery personnel, per 10,000 people	WHO Global Health Observatory, National Health Workforce Accounts [16]
Money going private	Voluntary health insurance plus household out-of-pocket payments, as a share of current health spending, same year	WHO Global Health Expenditure Database, final years only [17]
Long waits	Median days from specialist assessment to treatment, hip and knee replacement	OECD Health Statistics, waiting times [18]
A record that stops at the private door	Whether private providers write to the record the public system uses: connected, partial, split or unknown	Our country files, and outside research with a quoted source

Every value carries its own year, source and retrieval date. Years differ by country, from 2010 to 2025, and the expenditure data stop at 2023, the latest year the WHO database marks as final. Data were retrieved 2026-10-02. The wait measure counts the wait of patients who were treated, which understates the queue of those still waiting.

9.3 Flags

There is no composite score. Each country gets four flags, each true, false or unknown. A flag is true when doctors or nurses fall below the lower quartile, or private spending above the upper quartile, of the OECD members with a value (a fixed reference group, so the cut-offs do not move as countries are added): 30.76 doctors or 62.16 nurses and midwives per 10,000 (lower quartiles of the 38 and 38 OECD members with a value), and 28.66% of current health spending (upper quartile of 35). A wait flag is true when the longer of the hip and knee medians is over 90 days, the OECD's own three-month line. A record flag is true when the class is split. Unknown is never counted as false.

9.4 Coverage

Countries with a value, of 198: doctors 192, nurses and midwives 192, private insurance plus out-of-pocket spending 183, duplicate private insurance 19, median hip and knee waits 19. Flags: staffing 192 measured, 154 true; private spending 185 measured, 115 true; waits 19 measured, 14 true; record split 51 known, 5 true. Waits are reported for only 19 of 198 countries, mostly those that run waiting lists, and most of those exceed 90 days, so the wait flag separates little. Because the cut-offs are set by the OECD members, most countries outside the OECD carry the staffing flag, so it separates countries mainly within the OECD. Whether the record reaches private providers is known for 51 of 198. Of the known classes, 15

from our country files alone, 30 from the outside research alone and 6 from both, in agreement. Each class from the outside research rests on one quoted sentence from a named source, mostly laws, ministries and system operators. Each quote was fetched again and checked against its page, except on two pages that block automated access.

9.5 What it shows

By record class: 10 connected, 36 partial, 5 split and 147 unknown. Split means private providers do not write to the record the public system uses. Partial means some do, or some services do: often the gap is a single kind of provider, such as private imaging in one province. Counting both, in 41 of the 51 countries we could check the public health record does not fully reach private care; most of those are partial, not split. Chile, Mexico and South Africa carry three or more of the four flags; waits are not reported for 2 of the 3.

Canada, a worked example. Canada has 28.54 doctors per 10,000 (WHO, 2024), 31 of 38 OECD members (1 = most), and 116.18 nurses and midwives (14 of 38). Its median waits from specialist assessment to treatment are 120 days for a hip and 146 for a knee (OECD, 2025), 9 of 18 and 12 of 18 reporting members (1 = longest). Voluntary insurance pays 12.66% of health spending (2023), 2 of 35 OECD members with a value, and 67% of people hold it (2025, provisional); OECD reports no figure for duplicate cover. The record-split class is partial: British Columbia Ministry of Health, Digital Health Initiative: Health Gateway 'Diagnostic Imaging Reports' frequently asked questions states "Diagnostic imaging reports from most private clinics will not be available in Health Gateway as they currently are not available to our provincial repository." Canada carries 2 of 4 flags, and all four were measured. Canada carries 2 of the four flags: its doctor count (28.54 per 10,000) is below the OECD lower quartile (30.76) and its median hip and knee waits exceed 90 days. Its record reaches some private providers and not others, province by province and service by service. The data does not show waits driving people into a parallel private system for the same care: duplicate private insurance, the kind that buys a faster route to care the public plan already covers, has no OECD figure for Canada. The countries where OECD reports the most duplicate cover are Israel (87.6% of people; record class unknown), Ireland (46% of people; record class partial) and Australia (45.4% of people; record class partial).

9.6 Guards against reading it as a ranking

These are the layer's design rules for the interactive index.

- No composite and no flag-count column. Countries are listed alphabetically, never by flags.
- No color, and no mark on the globe, the map or the ranked table, so the layer cannot read as a second score.
- Unknown is printed as the word “unknown”, so missing data cannot pass for low strain.
- The quartiles across all countries mostly sort by national income. For OECD members we show the OECD rank beside each value.
- Voluntary insurance excludes compulsory private insurance, which is large in the United States, the Netherlands and Switzerland. It is shown separately.
- We do not correlate the record class with the journey or clinician scores. Where the class comes from our own country files, the two share a source and would agree by construction.

9.7 Status of the hypothesis

The hypothesis is not tested here. Public data can measure each link: staff, waits, private spending, and whether the record reaches private providers. It cannot test the causal step, that waits push people to private care and that this care then splits their record. That would need data on who goes private and why, and on what happens to their record when they do.

10. The traveling patient: does the record cross a border?

10.1 The question

The scores ask what a person can do with their record at home. People also travel, and some move. This section asks what happens to the same record at a border: when a person sees a doctor in another country, does any part of the record get there? It changes no score. It uses the same data as the interactive page at <https://healthrecordrights.com/traveler/>, and every number below is computed with that page's own code.

We follow five parts of the record: a short health summary, medicines (prescriptions), lab results, scans and X-rays, and hospital notes (what a hospital writes when a person goes home). Border status was checked on 2 October 2026.

10.2 How a part reaches a doctor abroad today

A part goes to a doctor in another country by itself only when both countries use the same live link. In Europe that link is MyHealth@EU, which the Commission runs to send a health summary or a

prescription from one country to another. It is “open to all the EU countries”, and it offers those two services only [37]. Lab results, scans and X-rays, and hospital notes are not among them. Joining is voluntary until the EHDS obligations apply [38].

A part counts as arriving by itself when an official source shows the service working between the two countries, or when the Commission’s monitoring figures show records being sent [39]. It does not mean every pharmacy or hospital uses it. Table 10 counts, among the 30 EU and EEA countries, those that send and receive each part.

Table 10. Countries where each part of the record is live on MyHealth@EU. Send: a doctor abroad can get this country’s record. Receive: a doctor here can get a visitor’s record from home. “Not carried”: MyHealth@EU has no such service.

Part of the record	Countries that send it	Countries that receive it
Health summary	11	15
Medicines (prescriptions)	12	12
Lab results	not carried	not carried
Scans and X-rays	not carried	not carried
Hospital notes	not carried	not carried

In all, 18 countries (17 EU members plus Norway) run at least one service in at least one direction. Only 9 (Croatia, Cyprus, Czechia, Estonia, Finland, Greece, Latvia, Portugal and Spain) send and receive both parts. A part needs a sender at one end and a receiver at the other. Count every one-way route between two of the 27 EU members, so that Spain to France and France to Spain count as two, and there are 702. A health summary travels by itself on 143 of them. Checking the summary and medicines on every route, 491 of the 1,404 checks (35%) are not known.

Outside the EU and EEA, 4 of the 168 countries take part in a live arrangement: Indonesia, Malaysia, Oman and Saudi Arabia, through the Hajj health card, a summary pilgrims carry as a QR code. For every other country we found no link.

10.3 Visitors and people who move

Every source we opened describes MyHealth@EU as a service for people who travel. The Commission says it ensures continuity of care for European citizens “while they are travelling abroad in the EU” [37]. The guideline the EU countries wrote for the health summary names two cases, the occasional visitor and the regular visitor who lives in one country and works in another, and describes only the first [40]. The prescription guideline is for “patients who are travelling inside Europe” [41].

No source says the service also works for people who move. None says it excludes them. So for all 30 EU and EEA countries, both parts are “not stated” for a person who has moved, and we draw them as not known, never as no. We found no way a record passes from the old country’s system to the new one. When a person moves, the country they leave keeps the records it made, and the new country starts its own. The record ends up in pieces, one in each country the person lived in.

10.4 The copy a person can bring

People can bring a copy. A right to a copy of the record applies in 138 of 198 countries, in a law, regulation or code their country file cites. Of the others, 23 say there is none and 37 do not say. In all 30 EU and EEA countries the GDPR gives a copy, and for a request made by electronic means, a copy “in a commonly used electronic form” [42]. In the United States, the HIPAA rule gives an electronic copy on request [43].

The page draws “you can bring a copy” only where the law says the copy can be electronic: 59 countries. Where a law gives a copy and says nothing of its form, the page says not known. We do not know whether a clinic abroad will accept or read a copy.

10.5 What EU law adds, and when

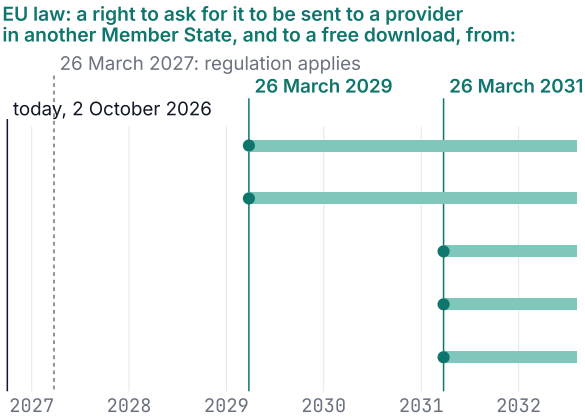
The European Health Data Space Regulation, (EU) 2025/327, applies from 26 March 2027 [5]. Its patient rights come later, in two steps set by Article 105. From 26 March 2029 they cover patient summaries, prescriptions and dispensations (Article 14(1), points (a) to (c)). From 26 March 2031 they cover medical images, test results and discharge reports (points (d) to (f)).

Article 7(2) gives a person the right to ask for their record to be sent to a provider in another Member State, through MyHealth@EU. The receiving provider “shall accept such data and shall be able to read them” [5]. So the right to ask runs from 26 March 2029 for the health summary and medicines, and from 26 March 2031 for labs, scans and X-rays, and hospital notes. Article 3(2) adds a free download in the European exchange format, on the same dates. The regulation’s own explanation names people who “change their place of residence” (Recital 33) [5].

These dates bind the 27 EU members. Whether they bind Norway, Iceland and Liechtenstein depends on the EEA Agreement taking in the regulation, which we did not check. A right in law does not move a record today.

Part of the record

Today, on MyHealth@EU
(2 October 2026)



Health summary patient summaries, point (a)	● live: 11 send, 15 receive
Medicines (prescriptions) ePrescriptions and eDispensations, (b) and (c)	● live: 12 send, 12 receive
Lab results test results, point (e)	○ not carried
Scans and X-rays medical images, point (d)	○ not carried
Hospital notes discharge reports, point (f)	○ not carried

Today: countries live among the 30 EU and EEA countries (Table 10); MyHealth@EU carries no other part. Dates: Regulation (EU) 2025/327, Article 105 with Articles 3(2), 7(2) and 14(1); they bind the 27 EU members (EEA take-up not checked). A right in law does not move a record today. Source: Health Record Rights Index, SuperTruth; analysis/crossborder.

Figure 9. What EU law adds for each part of the record, and when. Today MyHealth@EU carries only the health summary and prescriptions, between the countries that have joined. The EHDS gives a right to ask for each part to be sent to a provider in another Member State, and to a free download, from the dates shown.

10.6 Three worked examples

These are the three made-up trips on the interactive page. A piece is one part of the record, from one country, at one stop.

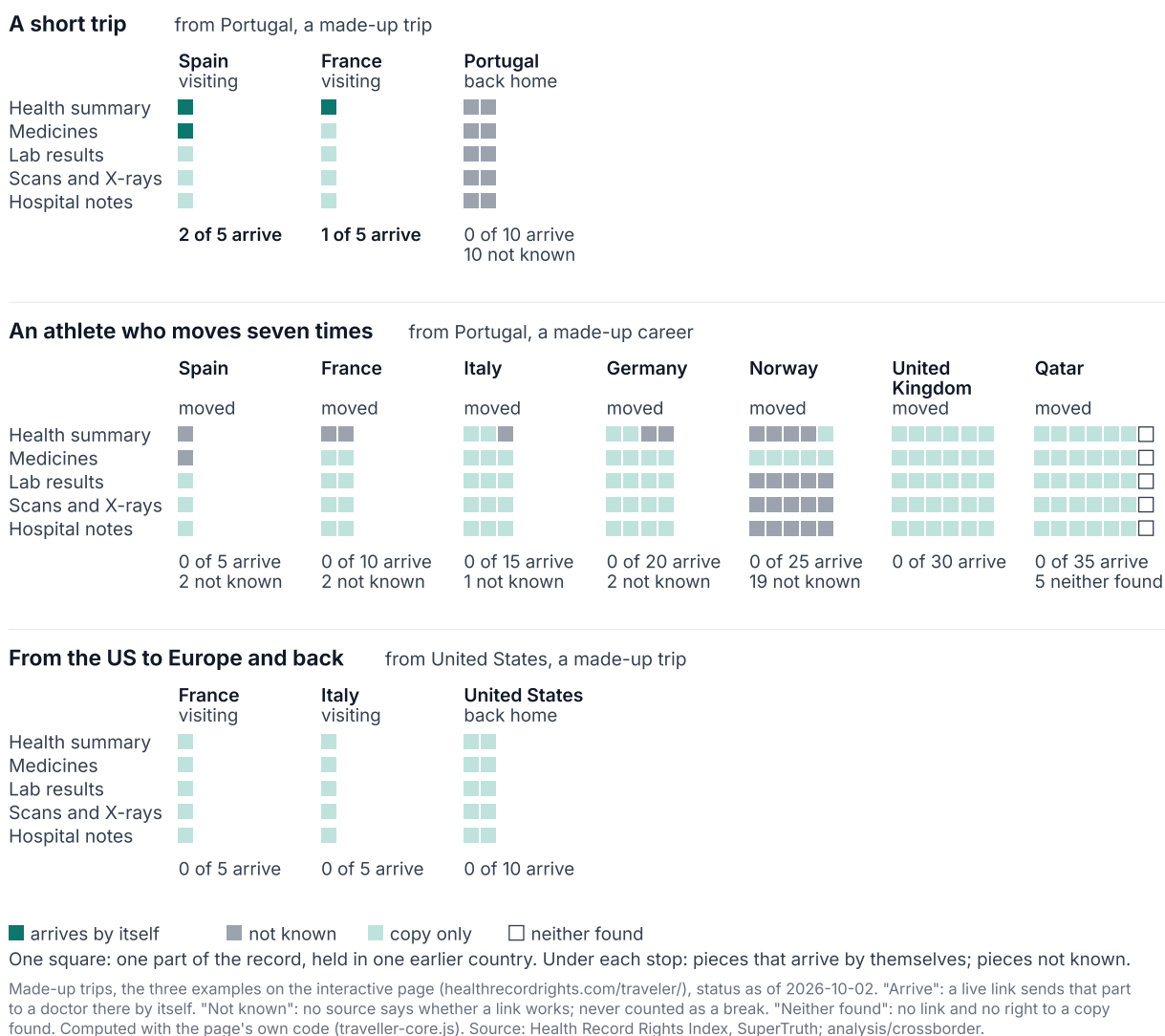


Figure 10. The three worked trips, stop by stop. Each square is one part of the record held in one earlier country. On the short trip some parts reach a doctor in Spain and France by themselves; on the athlete's career and the US trip none does, and with every move the record splits into more pieces.

A short trip. Portugal, then Spain (visiting), France (visiting) and Portugal (back home). In Spain, 2 of the 5 parts reach a doctor by themselves; in France, 1 of the 5 parts reaches a doctor by itself. Back home, no source says whether the care given abroad reaches the home record (10 of 10 pieces not known). Over the trip, no source says either way for 10 of the 20 pieces. Wherever a part does not arrive by itself, the person can bring a copy.

An athlete who moves seven times. Portugal, then Spain (moved), France (moved), Italy (moved), Germany (moved), Norway (moved), the United Kingdom (moved) and Qatar (moved). Each move adds a piece of the record in a new country. Along the way we check 140 pieces, and at none of the seven clubs does any part arrive by itself. At the last club, in Qatar, we found no part that reaches a doctor by

itself. For 26 of the 140 pieces (19%), no source says either way. Of those, 6 are a health summary or medicines that would arrive on a visit, where no source says they do for someone who moved there. At the four EU clubs (Spain, France, Italy and Germany) there are 50 pieces. For 7, no source says either way; 43 are not working yet, though a law says they must by 2029 or 2031. After each move between two EU countries, EU law gives a right to ask for the record to be sent, from 2029 for the health summary and medicines, and from 2031 for labs, scans and X-rays, and hospital notes. The athlete can bring a copy for 135 of the 140 pieces.

From the US to Europe and back. The United States, then France (visiting), Italy (visiting) and the United States (back home). In France and Italy, we found no part that reaches a doctor by itself. Back home, we found no part of the care given abroad that reaches the home record by itself. The EU law does not apply on this trip. Under the HIPAA rule the person can bring a copy of all 5 parts [43], and under the GDPR, a copy of every part of the care given in France and Italy [42].

10.7 Limits

- **Not known is not no.** A part is not known when no source says either way, or when sources disagree. We keep the disagreements: in 3 (Hungary, Lithuania and Poland), a national page says a service is planned or not working while the Commission's figures show records sent. Not known never counts as a break.
- **Old figures.** The Commission's monitoring figures end on 30 June 2025 and are under revision [39]. Where they are the only evidence, the status holds only to that date.
- **Live is not everywhere.** A live service runs between two countries. It need not reach every doctor or pharmacy, and we do not know how many it reaches.
- **Three parts not researched.** We did not research lab results, scans and X-rays, or hospital notes country by country. Inside the EU we show them as not yet, with the date the law sets.
- **"None found" is a weak negative.** Outside the EU and EEA it rests on one search per country and the Commission's own list, so a small pilot could be missed.
- **No outcomes.** We tested no exchange and counted no person whose record failed to arrive. Status as of 2 October 2026.

11. The 50 states and Washington, DC

This section is new in version 1.1. It is a layer of legal profiles, not scores. It gives no state a score, a total or a rank, and it does not change the national US score. It is not legal advice: every cell records what a primary legal text said on the date it was read, 3 October 2026, and laws change.

11.1 The question

The index scores the United States as one country, on federal law and national infrastructure. Yet much of what a person in the United States can do with their own record is set by the state they live in. Federal law sets a floor. HIPAA gives a right to a copy within 30 days, with one 30-day extension, at a reasonable cost-based fee (45 CFR 164.524). It lets providers and health plans share records for treatment, payment and health care operations without the person's consent (164.506). It does not reach most apps, wearables or data brokers. A state law that gives the person more than HIPAA stands; HIPAA does not override it (45 CFR 160.203). The information-blocking rule (45 CFR Part 171) adds to that floor for electronic access.

This layer asks one question of each state, and of Washington, DC: what does its own law add on top of federal law? Washington, DC is not a state. It has its own code, so it was researched in the same way, but it is reported on its own line and is never counted as a state. US territories and tribal law are out of scope.

State laws add to the federal floor. The national score rates federal law and national infrastructure and does not change with these state profiles: the US overall score stays as in Table 2, and its category scores as quoted in Section 7.3.

11.2 What we recorded

The rubric (`analysis/us/states/RUBRIC_STATES.md`) has seven dimensions: six take a level or a category, and one, D4 on sensitive records, has four yes-or-no parts. Each is a level or a category, never a 0 to 100 score, because each asks whether a provision exists and what it requires. A higher level means the provision gives the person more on that one question. It does not mean the state is better overall.

- **D1 Health data outside HIPAA** (levels 0 to 3). What state law requires before a business that HIPAA does not cover collects, shares or sells health data. Level 0 is nothing beyond general consumer law, level 1 is an opt-out only, level 2 is opt-in consent, and level 3 is a dedicated

consumer health data law that also needs a separate signed authorization to sell.

- **D2 A copy on better terms than federal law** (levels 0 to 2). One step each for a deadline shorter than 30 days and for a copy that must be free in at least one defined case. By an author ruling made before any research (change-log entry D-1 in `analysis/us/states/DEVIATIONS.md`), the floor here includes the information-blocking rule, so a free electronic copy that federal law may already require earns nothing.
- **D3 Consent rule for health information exchange (HIE)**, a network that passes records between providers (categories, not ordered): opt-in, opt-out, no choice required, or no rule found. Opt-in and opt-out trade the person's control against a connected record, so the layer does not rank them.
- **D4 Sensitive records** (four parts: reproductive health, mental health, HIV and genetic information). Whether a statute or regulation in force sets a confidentiality or disclosure rule for that kind of record. The rubric bars any word that grades a state on D4, and D4 records nothing about whether any care is legal or available.
- **D5 Right to sue** (levels 0 to 2). Level 1 is a right limited to a breach or to one narrow kind of record; level 2 is a right to sue over unlawful use or disclosure generally.
- **D6 Health data in the breach notification law** (levels 0 to 2). Whether medical information and health insurance information are in the law's definition of personal information.
- **D7 Rules for AI in care or coverage** (levels 0 to 3). One step each for a duty to tell the person that AI is used, a rule that a clinician decides, and a right to have a person review the decision.

Only a provision in force earns a level above the lowest. A law that is signed but not yet in force is recorded with its date and earns nothing until then. "No law found" is a finding, backed by a search log. "Not checked" means the question was not settled. The two are never counted together or drawn alike.

11.3 How the layer was built

Written down first. The plan, the rubric and every script were hashed and committed on 3 October 2026, before any state was researched (`analysis/us/states/PLAN.md`). Every later change is logged with its date and a number (D-20, D-21 and so on) in `analysis/us/states/DEVIATIONS.md`. The changes made during the run are listed in Section 11.7.

One agent per state. Each state, and Washington, DC, was researched by a fresh agent session. It started in an empty folder with no access to our repository, and it could only search the web, open pages and write one file. Evidence had to be a primary source opened in that session: the legislature's code, the regulator, or, for D3, the state-designated exchange's own policy. Each cited provision carries a quote of 50 words or fewer. Law-firm pages, legal databases, trade press and Wikipedia could point to a law but could not support a cell, except where an author ruling accepted a court order found only there (Section 11.7). Our harness (the script that launched the sessions and checked their output) then fetched every cited page itself, stored the bytes, and checked that each quote appears in the page text. For 17 quotes, on pages the harness could not read as text, that check was not possible; fresh agent sessions re-read them in the live pages and confirmed 17 of 17 (D-28), an agent reading rather than a stored-page match.

A cross-check, and a test of the cross-check. For each state a second fresh session opened every cited page again and confirmed or disputed each cell. To test whether it read the law or simply agreed, we planted one wrong cell in the copies it read for 23 states and Washington, DC. A plant was one of six kinds: a law relabeled "no law found", a level raised by one, a future law shown as in force, one word changed in a quote, a secondary page (such as a law-firm summary) swapped in for the official one, or another state's statute cited. The plan required the cross-checker to dispute at least 22 of the 24. It disputed 24 of 24 (the lower bound of the 95% interval is 85%). Our own verifier would catch the last two kinds anyway, and most plants of the third and fourth kinds were disputed on grounds the verifier also checks (a date after the reading date, a quote not found word for word). The plants that only a reading of the law could catch, a law relabeled "no law found" and a level raised by one, were disputed in 8 of 8 cases. That is a small test. One of the 24 verdicts, New Jersey's, came only from a third cross-check attempt. The first two attempts failed the transcript audit over how they wrote that state's web addresses, and the authors allowed a third (D-23). Without it the count is 23 of 24 (lower bound 78%), which still meets the threshold. Every planted cell was restored from its record before the files were put together.

Audits. Every session's transcript was audited by a script: only the allowed tools, no visit to our site or repositories, nothing read outside the session's own folder, and every cited page fetched in that session before its file was written. A verifier checks every finished file: statuses and levels in range, no level without a law in force, every quote found in the stored page (except the agent-read quotes

above), no field that could hold a total or a rank, and no grading word on D4. It also checks that the country data and the US scores did not move.

11.4 How far a blind reading agrees, and what an open search finds

Blind re-score. A pre-registered sample of 150 cells (the plan asked for 152; two regional quotas had too few eligible cells), drawn by dimension and by Census region from the states and Washington, DC, was rated again by fresh sessions. Each rater got the rubric, the state, the dimension and the cited pages, and nothing else: no level, quote or note. In the first run, every rater session opened pages beyond the cited ones, some by searching the web. So the arm was run again on the same sample with the raters locked in by mechanism, not only by instruction: read-only tools, no network except the model's own interface, inside an operating-system sandbox (D-22). They read copies of the cited pages that our harness made on the rating date, not the live pages. After the author rulings (Section 11.7), the 52 sampled cells that the rulings changed were rated once more under the same lock.

Against the published cells, the intraclass correlation (ICC(2,1), absolute agreement) on the five dimensions with ordered levels (D1, D2, D5, D6 and D7) was 0.965 (95% CI 0.950 to 0.976), over 114 cells. The interval assumes the cells are independent; a bootstrap that resamples whole states gives 0.92 to 1.00. Of the 118 sampled cells with ordered levels, 4 are left out of this figure and of exact level agreement because the rater could not settle a level from the cited pages; status agreement counts them. Exact level agreement was 96.5% and status agreement 96.0%. On the exchange consent rule (D3) the rater matched the published value in 93.3% (kappa 0.77, n 15) of cells, and on sensitive records (D4) in 100.0% (n 16; every sampled D4 cell was "yes", so kappa is undefined). The lowest single dimension was 91.7% (D2 copy rights, n 24). With each dimension's average level removed, the correlation was 0.963. As a check of the rater against itself: 8 cells rated twice by different sessions, 7 identical. Before the re-rating, the three headline figures were 0.979, 98.2% and 98.0%, on 147 rated cells (3 could not be checked from the stored page). All four pre-registered thresholds were met (ICC at least 0.75, exact level agreement at least 70%, status agreement at least 90%, and at least 85% on D3 and D4), so, under the plan, every dimension it allows on a map is drawn in Figure 11. When the sample was drawn, 25 of its cells were under dispute. Before the rulings, 3 disagreements met the plan's rule for referral and went to the editors (the authors' review step, Section 11.7). After the re-rating, 6 sampled cells meet that rule and are open, not yet referred: Iowa health data

outside HIPAA (D1), published in force (level 1), rater not checked; Louisiana AI in care (D7), published in force (level 0), rater not checked; Oklahoma AI in care (D7), published in force (level 1), rater not checked; Alabama AI in care (D7), published in force (level 1), rater not checked; Hawaii AI in care (D7), published in force (level 0), rater no law found (level 0); and New Jersey HIE consent (D3), published not checked, rater in force (opt-out). These figures compare the raters with the cells as published on the study's run date. One sampled cell has changed since: New Jersey's exchange consent, then not checked, now reads no rule found (D-35, Section 11.7). The rater read opt-out; the authors have since ruled that no rule found stands (Section 11.7).

These figures say how consistently two agent readings of the same cited evidence agree. They do not say whether the right law was found, because a rater who reads only the cited pages cannot see a law that was missed. The sample leaves out "no law found" cells, which have no page to re-read, and most sampled cells sit at the same few levels, which can make agreement look easier. The rater is the same model family as the researcher and the cross-checker, so they may share blind spots.

Open search. To look for missed laws, other fresh sessions got only the rubric, the state and the dimension, and searched on their own. The plan asked for 28 cells; 25 could be drawn, because some dimensions had fewer "no law found" cells than the plan asked for. At the time of the run, status agreed in 76.0% of cells, and where both found a provision the levels agreed in 84.6% of the 13 (86.7% of the 15 against the published cells after the rulings). In 4 cells the open search reported a law in force where the researcher had found none: Alaska copy rights (D2); Florida reproductive records (D4); Idaho copy rights (D2); and Alabama right to sue (D5). The plan's trigger for a second search was three such misses, so one was run.

A second search for missed laws. Every "no law found" cell in the 3 dimensions with a miss got its own fresh search session (D-24): 14 cells. In 8 the second search also found nothing. In 6 it reported a provision (5 in force and 1 signed but not yet in force). Each went to the editors with both readings; after the rulings, 4 of the published cells no longer read "no law found" (Section 11.7). Also after the rulings, 2 of the 4 open-search findings still differ from the published cell: Alaska copy rights (D2), published as no law found at level 0; and Idaho copy rights (D2), published as no law found at level 0. In Alaska the open search found a general right to a copy that adds nothing beyond federal law, so the level is the same either way.

In Idaho the open search read a 2024 edition of a hospital rule; the Idaho Administrative Bulletin records that rule as void from 1 July 2025.

11.5 Results

Table 11. The 50 states on six dimensions: how many states have each level or value in force; how many have only a signed law not yet in force, or a law stayed or repealed; in how many no law was found; and how many cells were not checked. Rows are not comparable with each other and are never added. Washington, DC is not in these counts; its line is below. Law read 3 October 2026.

Dimension (scale)	In force, by level or value	Only a signed law, not yet in force	Stayed by a court, or repealed	No law found	Not checked
D1 Health data outside HIPAA (levels 0 to 3)	0: 0; 1: 1; 2: 26; 3: 2	3	0	17	1
D2 Copy of the record on better terms than federal law (levels 0 to 2)	0: 13; 1: 27; 2: 8	0	0	2	0
D3 Consent rule for health information exchange (values, not ordered)	opt-out: 33; opt-in: 3; no choice required: 5	0	2	7	0
D5 Right to sue in one's own name (levels 0 to 2)	0: 5; 1: 31; 2: 11	0	0	2	1
D6 Health data in the breach notification law (levels 0 to 2)	0: 25; 1: 3; 2: 21	0	0	0	1
D7 Rules for AI in care or coverage (levels 0 to 3)	0: 6; 1: 12; 2: 6; 3: 1	2	0	21	2

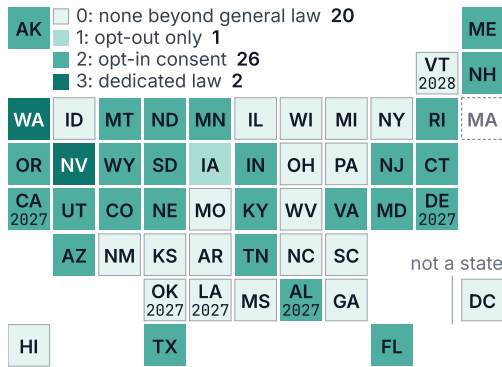
Table 12. The 50 states on sensitive records (D4): whether a confidentiality or disclosure rule for each kind of record is in force. These are not grades of any state, and D4 is never mapped or counted into anything else. Washington, DC is not in these counts.

D4 category	Confidentiality or disclosure rule in force	Only a signed law, not yet in force	No law found	Not checked
Reproductive health	49	1	0	0
Mental health	50	0	0	0
HIV	50	0	0	0
Genetic information	44	0	5	1

Washington, DC (not a state). D1 health data outside HIPAA, no law found; D2 copy of the record on better terms than federal law, level 0; D3 consent rule for health information exchange, opt-out; D5 right to sue in one's own name, level 1; D6 health data in the breach notification law, level 2; D7 rules for AI in care or coverage, no law found; and a D4 rule in force for all four categories.

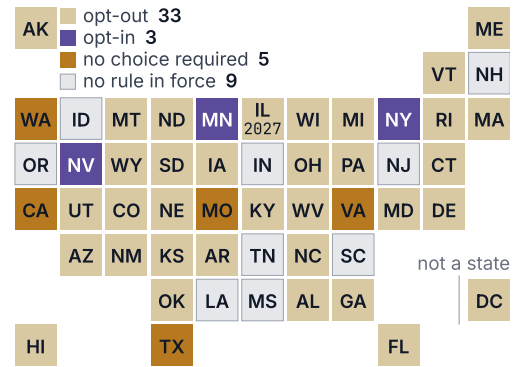
Health data outside HIPAA

Apps, wearables, brokers: what state law requires



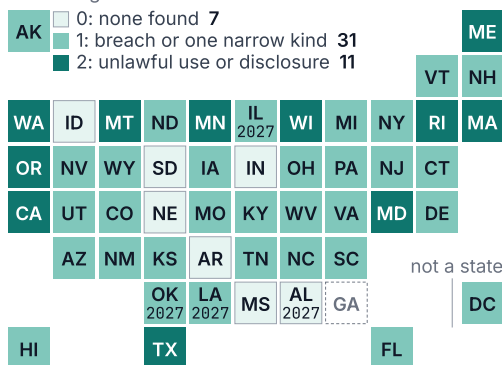
Consent rule for health information exchange

Values are not ordered



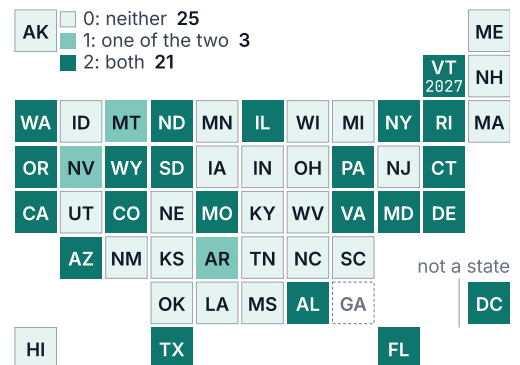
Can the person sue in their own name?

Private right of action



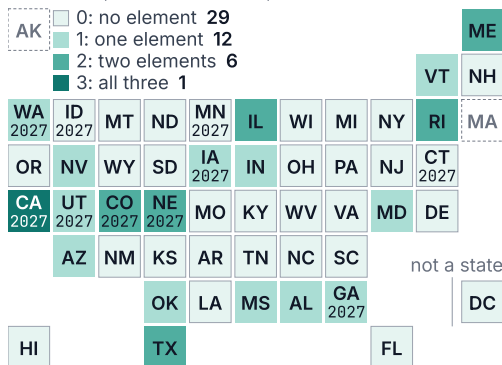
Health data in the breach law

Medical and health insurance information



Rules for AI in care or coverage

Disclosure, clinician decides, human recourse



One equal tile per state, placed roughly by geography; not an order. Shades read within one map only: maps do not share a scale.

Washington, DC (tile DC) is not a state: it sits apart and is not counted (its line: Section 11.5).

Counts are of the 50 states. Level 0 includes no law found and a law signed but not yet in force.

Copy rights and sensitive records: Tables 11 and 12 only; the plan does not allow a map of either.

Law read 2026-10-03. Not legal advice: every cell records what a primary legal text said on the date it was read. Source: Health Record Rights Index, US state layer (data/us-states), SuperTruth.

Figure 11. Five dimensions of the state layer, one map each; equal tiles placed roughly by geography, Washington, DC apart. Color is the law in force on the date it was read; a year on a tile marks a further law signed but not yet in force, dated under the map. Copy rights (D2) and sensitive records (D4) are in Tables 11 and 12, not mapped.

Read each row of Table 11 on its own; rows are not comparable and are never added. Table 11 counts a signed law only where nothing is yet in force, so it shows fewer signed laws than Figure 11 shows dated tiles. For health data outside HIPAA, 28 states require opt-in consent for at least some of it (level 2 or 3). That includes laws limited to one kind of data, such as genetic test results, or to insurers, which count under an author ruling (Section 11.7). Of those, 2 have a dedicated consumer health data law that needs a signed authorization to sell. In 17 we found no such law, and in 3 one is signed but not yet in force. On copy terms, 35 states give at least one copy term better than federal law, and 8 give both a shorter deadline and a free copy. Exchange consent is opt-out in 33 states and opt-in in 3; every state's answer is checked. On the right to sue, 11 states let a person sue over unlawful use or disclosure generally, and 31 only after a breach or for one narrow kind of record. On breach laws, 21 states name both medical and health insurance information in their breach law, and 25 name neither. On AI in care or coverage, 19 states have at least one rule in force for AI in care or coverage, 1 has all three elements, and in 21 we found none. The sensitive-record rules in Table 12 vary little, as the plan expected.

In 19 states at least one relevant law is signed but not yet in force, with dates from 1 January 2027 to 1 January 2028. Each earns nothing until its date, and the plan requires a re-check within 14 days after it. Of the 500 state cells, 6 are not checked. Of the provisions behind the cells, 15 carry the flag "current version date not confirmed" and 2 carry the flag "court order not yet pulled from the court record" under author rulings (Section 11.7). No cell is marked under dispute, but the blind re-score leaves some open (Section 11.4).

11.6 Rights in practice: context, not scored

The legal profile says what the law offers, not whether people are in a position to use it. Beside the legal profile sits a separate strand of public statistics, arranged under the domains of social determinants of health in the US government's Healthy People 2030. It is not scored or weighted, and it is never mixed into a legal dimension or used to account for one. We draw no causal link between Table 13 and Tables 11 and 12. The values were drawn through DataSpine, a data service of Artists, Robots Collective, LLC (see Conflict of Interest), on 4 October 2026, and each was checked against its publisher's own interface (the Census Bureau or the Bureau of Labor Statistics): 867 checks, 0 mismatches. Only state-level figures are used. In all, 7 planned measures are not shown: for some the only source is county-level, and a state figure would be a new estimate we have not validated; for the rest no state-level source was found.

Two counts are held but not shown, people in poverty and people with a bachelor's degree: the data service has no matching denominator for a share, so the education domain shows no figure. No state-level source on patient portal use was found.

Table 13. Context, not scored. Sources: Census Bureau Small Area Health Insurance Estimates (SAHIE) and American Community Survey (ACS); Bureau of Labor Statistics Local Area Unemployment Statistics (LAUS). The lowest, median and highest value across the 50 states, with Washington, DC on its own. No state is named and the table is not a ranking. Each figure carries its source and the period its data cover: the American Community Survey figures are 5-year estimates for 2018 to 2022, not one year.

Measure	Source, period	50 states: lowest	Median	Highest	Washington, DC (not a state)
Uninsured, under 65 (%)	Census SAHIE, 2024	3.3	8.7	19.1	4.8
Households with a broadband subscription (%)	Census ACS 5-year, 2018 to 2022	80.0	88.0	92.1	89.0
Households with a computer (%)	Census ACS 5-year, 2018 to 2022	88.8	93.5	97.2	95.4
Households whose only internet device is a smartphone (%)	Census ACS 5-year, 2018 to 2022	5.2	8.3	17.0	7.5
Median household income (US dollars of 2022)	Census ACS 5-year, 2018 to 2022	52,985	72,090	98,461	101,722
Unemployment rate (%)	BLS LAUS, December 2025	2.2	4.2	5.5	6.7
Households with no vehicle (%)	Census ACS 5-year, 2018 to 2022	3.8	6.2	29.0	35.7
People who speak a language other than English at home (%)	Census ACS 5-year, 2018 to 2022	2.5	11.8	43.9	17.5
People 65 or over living alone (% of households)	Census ACS 5-year, 2018 to 2022	7.5	12.1	14.6	10.6

11.7 Changes made during the run, and how disputes were ruled

The plan says that no change is made after results are seen. Several changes were. Every change from D-19 to D-35 in `analysis/us/states/DEVIATIONS.md` is listed here, with whether it was made after transcripts or results had been read.

- **Sessions voided by a billing error (D-19). Logged after those sessions ran.** The first research queue ended in a billing error on the API key before any session wrote a file; those sessions were voided, not counted as attempts under the plan, and the research was run again.
- **The transcript audit was replaced during the run (D-20). Made after the research transcripts were read.** The audit rules set before the run failed most of the first research sessions, almost all on harmless shell syntax; 23 passed. Running them again under the same rules would have failed them again. This reversed an earlier promise not to change audit rules during the run. Before the new audit was adopted, a separate agent session with no stake in it tried to break it and found

41 ways a real violation could pass. All were fixed and tested with planted transcripts. A second review was tried twice and both times the model stopped on a safety check before writing any test, so the adoption rests on the first review. The new rules read only the form of the commands a session ran, never what any cell says. Under them, all but 2 research sessions passed. The two that failed (Minnesota and Wyoming) broke real rules and were run again once.

- **Further audit fixes (D-20a to D-20c). Made after transcripts were read.** An edit of a session's own output after it was written now fails every audit. A cross-check session may read files it saved in its own folder. Web addresses are compared without regard to letter case, and the final address after redirects counts as fetched. Alabama and New Jersey cross-checks were run again under these rules.
- **State files held outside the published data until the authors ruled (D-21).** Not a change to any finding.
- **Blind raters locked to stored copies (D-22). Made after the first blind run was read.** The whole arm was run again on the same pre-drawn sample.
- **Files that the agent tool writes for itself kept in each session's own folder; a third New Jersey cross-check; the rater audit moved onto the new rules (D-23). Made after transcripts were read.** The third New Jersey attempt went beyond the plan's two, by author ruling.
- **The second search for missed laws (D-24).** The pre-registered rule set it off. Its format, one fresh session per cell, was decided after the open-search results were known.
- **Editor rulings drafted by an agent and approved by the authors (D-25). Made after results were known.** In the words of the run record (analysis/us/states/main/RULINGS_METHOD.md): "The editor rulings for the state layer were drafted by an agent against the primary sources and approved by the authors (J. A. Snyder, in consultation with co-authors R. P. Hill IV and D. Raney; 3 October 2026). Judgment calls were held back from that approval and ruled by the authors separately (DEVIATIONS D-26, D-27 and D-29; who ruled: D-31)." An earlier version of that record said judgment calls would go to named outside reviewers; none was contacted, and the authors ruled them (D-31 corrects the record). In all, 252 proposed rulings (rows in the rulings file) were drafted: the editor queue (219 rows on 146 cells), the planted cells and the unplanted New Jersey cells. Of these rows, 146 were drafted by an agent against the law's text and approved by the authors, 98 rows on 71 cells were held back as judgment calls and ruled by the authors separately, and 8 needed no ruling. A row could go into that approval only if it was mechanical, rested on a primary-source quote that passed the quote check, and changed no level or value; none of these approved rulings changed a

level or value. The delegated rows were approved as a set. Each ruling was turned into exact edits, applied only if every edit matched the file's current text, and logged with its before and after values; 82 of the applied edits are notes or citations that an agent wrote from a ruling's instruction rather than text copied from the ruling, and the log flags each one. Whether a quote passed its check was decided by our harness, never by a ruling.

- **Author rulings (D-26, D-27, D-29). Made by the authors after the results were seen, and not delegated:** J. A. Snyder decided, in consultation with R. P. Hill IV and D. Raney. The authors answered general questions on how the rubric applies and then accepted every remaining recommendation the drafting agent had made: a consent law limited to one kind of data, or to insurers, counts on D1; payment data counts as health insurance information on D6; a provision with verified text but no official page dating its current version keeps its value and carries the flag "current version date not confirmed"; where a state's statutes set a default effective date, that date is used unless the act names its own; a law in force that earns nothing is shown as in force at level 0 when its date is known; and a court order found only on a law-firm or press site counts, with the provision marked "court order not yet pulled from the court record". The last departs from the rubric's own evidence rule for the cells it touches. On 4 October they ruled on the pre-publication findings, including that exchange consent in Massachusetts is opt-out under its regulation. The author rulings changed the level or value of 19 cells: 6 raised (North Dakota health data outside HIPAA (D1), 0 to 2; South Dakota breach law (D6), 1 to 2; Texas breach law (D6), 1 to 2; Georgia AI in care (D7), 0 to 1; New York right to sue (D5), 0 to 1; and Delaware copy rights (D2), 0 to 1); 2 lowered (Colorado AI in care (D7), 3 to 2; and Louisiana AI in care (D7), 1 to 0); 6 set to not checked (Massachusetts health data outside HIPAA (D1); Massachusetts AI in care (D7); New Hampshire HIE consent (D3); Alaska AI in care (D7); New Jersey HIE consent (D3); and Connecticut HIE consent (D3)); 1 moved from not checked to a finding (Wyoming copy rights (D2), level 0); and 4 changed value (Massachusetts HIE consent (D3), opt-in to opt-out; Florida reproductive records (D4), no rule in force to a rule in force; Washington, DC genetic records (D4), no rule in force to a rule in force; and Missouri HIE consent (D3), no rule found to no choice required). Rulings also changed the status of 2 cells without changing a level or value: New Hampshire reproductive records (D4), no law found to enacted not yet effective; and Alabama right to sue (D5), no law found to in force. Two provenance choices follow from the rulings: an Alabama mental health date is taken from an archive.org scan of the official 1995 Acts volume (D-29), not from a

state page; and a Kansas reproductive records cell keeps the researcher's reading, dated by a 2024 amendment that the cross-checker reported a court has enjoined, with no flag (D-27). The Kansas value does not depend on that amendment.

- **Pre-publication checks (D-28). Made after results were known.** Fresh sessions searched for missing dates and re-read quotes that could not be matched to a stored page; a date was applied only with an official page and a quote our harness found on it, except the Alabama date above. The 52 sampled cells the rulings had changed were re-rated (Section 11.4). The data service corrected statistics that our plan had excluded because they failed a check against the Census Bureau; the corrected figures are used only where our own build matched them to the Census Bureau's. A harness bug gave the check sessions tools they did not need; none used them, and all were run again.
- **Display corrections (D-30). Made after the rulings.** A note on Washington, DC's genetic records that still carried the researcher's first reading was rewritten to match the ruled value, and provisions that lacked a status field were given the status of their cell. No level, value or cell status changed.
- **Who made the rulings (D-31). A record clarification, no data change.** The author rulings and the approval of the delegated rulings were made by J. A. Snyder in consultation with his co-authors, R. P. Hill IV and D. Raney.
- **Alabama right to sue marked pending (D-32). An author ruling, made after the results were seen.** The cell lists a provision signed and in force from 1 May 2027 but was not marked pending, unlike every other such cell. It is now marked; its status and level (in force, level 0) do not change, and Figure 11 shows the year on its tile.
- **Who gets the faster copy deadline, in Florida and Delaware (D-33). An author ruling, made after the results were seen.** An answer-quality test of the data service found that Florida's 14-day deadline covers nursing-home residents only, and Delaware's faster deadline long-term care only. Each cell now says who the faster deadline covers. No level changed: Florida stays at level 2 and Delaware at level 1.
- **Texas copy deadline worded exactly (D-34). A display clarification.** Texas gives hospitals 15 days but physicians 15 business days; the cell now names both. No level changed.
- **Exchange answers filled in (D-35). An author instruction, made after the results were seen (J. A. Snyder, 5 October 2026: "fix the gaps").** It changed the consent rule for health information exchange only. The four cells left not checked by the author rulings above were researched again from primary sources. Alabama and Connecticut

are opt-out. New Hampshire's law still gives an opt-out from "the health information organization", but a 2020 law repealed that organization's definition, so the cell reads repealed, with no rule found. In New Jersey no rule was found: the state's exchange describes an opt-out, but we found no published way for a patient to opt out, so the rubric's opt-out test is not met. Indiana, Nevada and Ohio now name their exchange, and Louisiana, Mississippi, Missouri, South Carolina and Tennessee say plainly that none is designated, each with sources. No other answer changed. Each record of a change gives the answer before and after (analysis/us/states/main/hie_gaps_d35.json). On 6 October 2026 the authors ruled that New Jersey's and South Carolina's answers stand as recorded: in New Jersey, a described opt-out with no published way to use it does not meet the rubric's opt-out test; in South Carolina, no state source designates the exchange that publishes an opt-out.

These changes are why the agreement figures in Section 11.4 are reported both before and after the rulings.

11.8 Limits

- **Dates.** Every cell records what a primary text said on the date it was read. The plan calls for a full re-check every six months, and for a re-check of each signed law within 14 days after it takes effect.
- **One model family.** The research, cross-check, rating and ruling-drafting sessions all ran on the same model, so they may share blind spots. No lawyer has checked a cell, and no person rated the sample.
- **Completeness.** The blind re-score measures how consistently cited evidence is read. Only the small open-search arm and the second search look for missed laws.
- **Rulings after results.** The author rulings were made after the results were seen, and some raised or lowered a level (Section 11.7).
- **Flags.** Some provisions carry "current version date not confirmed", and some rest on a court order cited from a law-firm or press copy until the court record is pulled.
- **What a level means.** A level says what a provision requires, not whether it is enforced or obeyed. Two states at the same level can differ in ways the level does not show; the quoted provisions carry those differences.
- **Copies of pages.** The locked raters read copies of the cited pages made on the rating date.
- **Exchange consent in New Jersey and South Carolina.** Both read no rule found, by author ruling (Section 11.7). New Jersey's search of the state's code was a fallback, because the legislature's sites did not

respond on 5 October 2026; the legislature's own search is rerun at the next six-monthly re-check. In South Carolina, the SC eHealth Alliance calls itself the statewide exchange and publishes an opt-out, but no state source designates it.

- **Context.** The figures in Table 13 describe conditions, mostly from 2018 to 2022. None measures whether anyone used a record right.

12. A method note: counting posts on X

We tried to count posts on X (formerly Twitter) about real problems with health records, as a second view of what people report. We used the official X API (full-archive counts and search) for posts from 2024-10-01 to 2026-09-30, with one query per country in its main languages. Raw counts are not a measure: most matching posts are policy talk, news about other countries, spam or jokes. So an agent read a random sample of matching posts for each country and classified each against a written definition. A country's figure would be shown only if at least 60% of at least 20 sampled posts were on topic. Two deliberately broad test queries failed the gate, which shows the gate can reject a broad query.

Only 7 of 43 countries passed: Argentina, Chile, Germany, Mexico, Netherlands, Poland and Spain. Most of what passed was breach news, recall was not measured, and the figures could not be compared across countries because X use and query breadth differ by country. The 155 countries added after the run started were not measured. We therefore do not show X counts in the index. For most countries, public posts on X cannot be turned into a usable count of record problems. No post text, handle or post identifier is kept in the repository.

13. Authors' position: the record should travel with the person

This section is the authors' position, not a finding. It aligns with what SuperTruth sells (see Conflict of Interest).

More individual control is not free. In a randomized trial of 2,228 new outpatients at one Dutch tertiary hospital, an opt-out procedure for the secondary use of routine health data gave higher consent rates than opt-in, with less bias by gender, socioeconomic status and country of birth [26]. That trial concerns research use, not sharing in care, and one hospital. It still shows that a stricter consent default can make the data that research and planning rely on less

representative. The index's rubric rewards both the person's choice and research that remains possible (Section 2.1), and it does not yet say how to weigh one against the other.

The informatics literature also finds that personal health records give greater benefits when they are integrated with electronic health record systems than when they stand alone [54]. We take both points as limits on what follows.

All 9 countries in the Strong band are Shared: they give the person a register of consent, a log of who looked, and a copy of their own record, inside infrastructure that the state or providers run. That is a real achievement, and most countries in this index fall short of it. It is also not the same as the person holding the record.

As clinical AI reads more records, the question of who holds them becomes a question of who can check them. Records contain errors that the people they describe can see. In a survey at three US health systems, 21.1% of 22,889 patients who read their visit notes reported a mistake they perceived, and 42.3% of those called it serious [23]. Those are errors as patients perceived them, not errors confirmed by a clinician. A record the person can see is a record the person can correct. A record whose every use is logged is a record whose use can be proven. We think the next step for every system in this index, the strong ones included, is to make the record verifiable by the person it describes.

Funding

This work was supported by internal funding from SuperTruth Inc. No external funding was received.

Conflict of Interest

Three of the authors, Jason Alan Snyder, Robert P. Hill IV and Dustin Raney, are officers of SuperTruth Inc., which sells health data verification products. The fourth, Leann Sims, is Chief Executive Officer of MedSync, a US health information exchange that works with SuperTruth. SuperTruth funded this work and controlled its design, analysis, writing and the decision to publish. The evidence grade in Section 8 adapts SuperTruth's own Data Trust Index; its source [6] is the first author's own deposit and has not been peer reviewed. The position in Section 13 aligns with what SuperTruth sells. SuperTruth built this index and chose the accounts in Section 5 itself; no country, company or person paid to be included, excluded or scored. imaware, an affiliate of SuperTruth, provides testing

services under agreements with some US state governments. The context figures in Section 11.6 come from DataSpine, a data service of Artists, Robots Collective, LLC, a company co-founded by J. A. Snyder, who is its Chief AI Officer; every DataSpine value shown was checked against the Census Bureau or the Bureau of Labor Statistics. The index scores the United States as one country. Section 11 profiles the law of the 50 states and Washington, DC, including the states where imaware holds agreements. The profiles give no state a score or a rank, and no government or health system paid to be included or had any say in its scores or profiles. The state layer asks about consent for health information exchange; it records the exchange a state names, where there is one, but scores no exchange and does not name MedSync. No author or reviewer outside SuperTruth and MedSync took part. Readers should weigh this disclosure when evaluating the results.

Intended Use

This index is a research tool. It is not legal advice and does not assess the compliance of any organization. The accounts in Section 5 are summaries of published sources and make no finding of our own about any person or organization. The context layer in Section 9 is not a score and not a ranking. The state layer in Section 11 is not legal advice. It gives no score and no ranking: it records what a primary legal text said on the date it was read, and its context figures are never scored.

Data and Code Availability

The data are deposited on Zenodo;
<https://doi.org/10.5281/zenodo.23120172> always resolves to the latest data version, and data version 1.0 is at
<https://doi.org/10.5281/zenodo.23120173>. Version 1.0 of this paper is at <https://doi.org/10.5281/zenodo.23120175>, and
<https://doi.org/10.5281/zenodo.23120174> always resolves to the latest version. The public release is a clean export of the project, in a public repository linked from the Zenodo record. It holds the scores, category texts, sources, laws, journey maps and news items for every country; the rubric and its v1.1 anchors; the build script that computes every overall score; the audit and the record of score changes; the evidence grade; the strain and split layer; the external cross-check; the stability analysis and the blind re-scoring (plan, rater kit and ratings); and the script that fills every number in this paper. It does not include internal working files such as drafts, launch materials and internal review notes, and it does not include the

individual accounts. For about an hour on 2 October 2026 the public copy also held the accounts and the internal review notes, and for about two hours on the night of 2 to 3 October it held the review notes again; both times they were removed and the repository history replaced, but copies made in those windows cannot be recalled. The paper and data are released under CC BY 4.0 and the scoring and analysis code under the MIT license, copyright SuperTruth Inc.

The state layer is in the public export and in the Zenodo data record from data version 1.1 (<https://doi.org/10.5281/zenodo.23137895>). Data version 1.2 (<https://doi.org/10.5281/zenodo.23174624>), which this version of the paper uses, adds the D-34 and D-35 answers and the record of the D-35 changes. The deposit holds the files for the 50 states and Washington, DC (`data/us-states`) and, in `analysis/us/states`, the plan, rubric, log of changes, decision rules, rulings method, applied rulings and agreement results. The session transcripts and the stored copies of statute pages are kept out of every public export, because statute pages hosted by vendors carry their own terms.

The interactive index is at <https://healthrecordrights.com>. Each of the 198 countries also has a printable one-page brief at <https://healthrecordrights.com/brief/ISO3/> (for example `/brief/FIN/` for Finland). “Ask the index” is an agent on the same page that answers questions only from the index’s own data and says so when the index does not hold an answer.

Note added 5 October 2026: brief addresses now carry the country’s name, for example <https://healthrecordrights.com/brief/finland/>, and the US state pages the state’s name (`/us/states/ohio/`). The code form printed above, and in versions 1.0 to 1.2 of this paper, redirects permanently to the named address, so the published links keep resolving.

The accounts in Section 5 are linked from the interactive index and are not deposited with this paper, so that an account can be taken down from the index on request. Copies made while the accounts were public on 2 October 2026 cannot be recalled. Aggregate counts are in Tables 4 to 6. Removal and correction requests: see the interactive index.

Author Contributions

All four authors conceived the index together, directed the research, reviewed the results and approved the final text, and share equal credit.

About the Authors

Jason Alan Snyder is Co-Founder and Chief AI Officer of SuperTruth Inc. **Robert P. Hill IV** (Bobby Hill) is Co-Founder and Chief Executive Officer of SuperTruth Inc. **Dustin Raney** is Chief Strategy Officer of SuperTruth Inc. **Leann Sims** is Chief Executive Officer of MedSync.

References

- [1] European Commission (2025). *Digital Decade 2025: eHealth indicator study*. <https://digital-strategy.ec.europa.eu/en/library/digital-decade-2025-ehealth-indicator-study>
- [2] Slawomirski L, Lindner L, de Bienassis K, Haywood P, Hashiguchi T C O, Steentjes M, Oderkirk J (2023). *Progress on implementing and using electronic health record systems: Developments in OECD countries as of 2021*. OECD Health Working Papers No. 160. OECD Publishing, Paris. <https://doi.org/10.1787/4f4ce846-en>
- [3] Thiel R, Deimel L, Schmidtman D, Piesche K, Hüsing T, Rennoch J, Stroetmann V, Stroetmann K (empirica) (2019). *#SmartHealthSystems: International comparison of digital strategies*. Bertelsmann Stiftung. Summary (Focus Europe): https://www.bertelsmann-stiftung.de/fileadmin/files/Projekte/Der_digitale_Patient/VV_SHS_Europe_eng.pdf. Full study: https://www.bertelsmann-stiftung.de/fileadmin/files/Projekte/Der_digitale_Patient/VV_SHS-Studie_EN.pdf
- [4] World Health Organization. *Global Digital Health Monitor*. Data accessed 1 October 2026. <https://data.who.int/dashboards/gdhm/data>
- [5] Regulation (EU) 2025/327 of the European Parliament and of the Council of 11 February 2025 on the European Health Data Space and amending Directive 2011/24/EU and Regulation (EU) 2024/2847. OJ L, 2025/327, 5 March 2025. CELEX 32025R0327. ELI: <http://data.europa.eu/eli/reg/2025/327/oj>. In force 25 March 2025; applies from 26 March 2027, with the patient access rights (Articles 3 to 15) applying from 26 March 2029 or 26 March 2031 depending on the data category, and the secondary use chapter (Chapter IV) from 26 March 2029, with listed exceptions (Article 105).
- [6] Snyder J A (2026). *The Data Trust Index: A Multidimensional Framework for Evaluating Health Data Integrity in AI Systems*. Zenodo. <https://doi.org/10.5281/zenodo.19601616>
- [7] Essén A, Scandurra I, Gerrits R, Humphrey G, Johansen M A, Kierkegaard P, Koskinen J, Liaw S-T, Odeh S, Ross P, Ancker J S (2018). Patient access to electronic health records: Differences across ten countries. *Health Policy and Technology* 7(1):44-56. <https://doi.org/10.1016/j.hlpt.2017.11.003>
- [8] Kharko A, Blease C, Johansen M, Moen A, Scandurra I, McMillan B, Hägglund M (2024). Mapping patients' online record access worldwide: Preliminary results from an international survey of healthcare experts. In *MEDINFO 2023: The Future Is Accessible*. Studies in Health Technology and Informatics 310:114-118. IOS Press. <https://doi.org/10.3233/shti230938>
- [9] Hägglund M, Kharko A, Hagström J, Bärkås A, Blease C, et al. (2023). The NORDeHEALTH 2022 patient survey: Cross-sectional study of national patient portal users in Norway, Sweden, Finland, and Estonia. *Journal of Medical Internet Research* 25:e47573. <https://doi.org/10.2196/47573>
- [10] Hägglund M, Kharko A, Bärkås A, Blease C, Cajander Å, et al. (2024). A Nordic perspective on patient online record access and the European Health Data Space. *Journal of Medical Internet Research* 26:e49084.

<https://doi.org/10.2196/49084>

- [11] Moll J, Scandurra I, Bärkås A, Blease C, Hägglund M, Hörhammer I, Kane B, Kristiansen E, Ross P, Åhlfeldt R-M, Klein G O (2024). Sociotechnical cross-country analysis of contextual factors that impact patients' access to electronic health records in 4 European countries: Framework evaluation study. *Journal of Medical Internet Research* 26:e55752. <https://doi.org/10.2196/55752>
- [12] Munung N S, Staunton C, Mazibuko O, Wall P J, Wonkam A (2024). Data protection legislation in Africa and pathways for enhancing compliance in big data health research. *Health Research Policy and Systems* 22:145. <https://doi.org/10.1186/s12961-024-01230-7>
- [13] Townsend B A, Sihlahla I, Naidoo M, Naidoo S, Donnelly D-L, Thaldar D W (2023). Mapping the regulatory landscape of AI in healthcare in Africa. *Frontiers in Pharmacology* 14:1214422. <https://doi.org/10.3389/fphar.2023.1214422>
- [14] Alegre V, Álvarez M Y, Bianchini A, Buedo P, Campi N, et al. (2024). Salud digital en América Latina: legislación actual y aspectos éticos. *Revista Panamericana de Salud Pública* 48:e40. <https://doi.org/10.26633/rpsp.2024.40>
- [15] Essén A, Stern A D, Haase C B, Car J, Greaves F, Paparova D, Vandeput S, Wehrens R, Bates D W (2022). Health app policy: International comparison of nine countries' approaches. *npj Digital Medicine* 5:31. <https://doi.org/10.1038/s41746-022-00573-1>
- [16] World Health Organization. *Global Health Observatory: National Health Workforce Accounts*, indicators HWF_0001 (medical doctors per 10,000) and HWF_0006 (nursing and midwifery personnel per 10,000), December 2025 update. https://ghoapi.azureedge.net/api/HWF_0001
- [17] World Health Organization. *Global Health Expenditure Database*, release of 12 December 2025. <https://apps.who.int/nha/database>
- [18] OECD. *OECD Health Statistics: Waiting times* (DSD_HEALTH_PROC@DF_WAITING) and *Healthcare coverage* (DSD_HEALTH_PROT@DF_HEALTH_PROT). <https://sdmx.oecd.org>
- [19] Björnberg A, Phang A Y (2019). *Euro Health Consumer Index 2018 Report*. Health Consumer Powerhouse. Grey literature. Opened through the Internet Archive: <https://web.archive.org/web/2019/https://healthpowerhouse.com/media/EHCI-2018/EHCI-2018-report.pdf>
- [20] Health Consumer Powerhouse (2009). *The Empowerment of the European Patient 2009: options and implications*. Grey literature. https://www.elpartoesnuestro.es/sites/default/files/2011/09/the-empowerment-of-the-european-patient_1.pdf
- [21] Nardo M, Saisana M, Saltelli A, Tarantola S (European Commission Joint Research Centre); Hoffmann A, Giovannini E (OECD) (2008). *Handbook on Constructing Composite Indicators: Methodology and User Guide*. OECD Publishing, Paris. ISBN 978-92-64-04345-9. <https://doi.org/10.1787/9789264043466-en>
- [22] Flemyng E, Noel-Storr A, Macura B, Gartlehner G, Thomas J, Meerpohl J J, et al. (2025). Position statement on artificial intelligence (AI) use in evidence synthesis across Cochrane, the Campbell Collaboration, JBI and the Collaboration for Environmental Evidence 2025. *Environmental Evidence* 14:20. <https://doi.org/10.1186/s13750-025-00374-5>. Published at the same time in the Cochrane Database of Systematic Reviews, Campbell Systematic Reviews and JBI Evidence Synthesis.
- [23] Bell S K, Delbanco T, Elmore J G, Fitzgerald P S, Fossa A, Harcourt K, Leveille S G, Payne T H, Stametz R A, Walker J, DesRoches C M (2020). Frequency and types of patient-reported errors in electronic health record ambulatory care

- notes. *JAMA Network Open* 3(6):e205867.
<https://doi.org/10.1001/jamanetworkopen.2020.5867>
- [24] Besley T, Hall J, Preston I (1999). The demand for private health insurance: do waiting lists matter? *Journal of Public Economics* 72(2):155-181.
[https://doi.org/10.1016/s0047-2727\(98\)00108-x](https://doi.org/10.1016/s0047-2727(98)00108-x)
- [25] Johar M, Jones G, Keane M, Savage E, Stavrunova O (2011). Waiting times for elective surgery and the decision to buy private health insurance. *Health Economics* 20(S1):68-86. <https://doi.org/10.1002/hec.1707>
- [26] Hermus M, Scharloo-Karels C H, Ikram M A, Andrinopoulou E-R, Rizopoulos D, Marck D H, Michels M, van Kemenade F (2025). Opt-in versus opt-out for the secondary use of routinely recorded health data: A randomized controlled trial. *European Journal of Internal Medicine* 133:100-105.
<https://doi.org/10.1016/j.ejim.2025.01.017>
- [27] Liu V, Musen M A, Chou T (2015). Data breaches of protected health information in the United States. *JAMA* 313(14):1471.
<https://doi.org/10.1001/jama.2015.2252> (corrected version).
- [28] Jiang J X, Ross J S, Bai G (2025). Ransomware attacks and data breaches in US health care systems. *JAMA Network Open* 8(5):e2510180.
<https://doi.org/10.1001/jamanetworkopen.2025.10180>
- [29] Richwine C, Johnson C, Patel V (2023). Disparities in patient portal access and the role of providers in encouraging access and use. *Journal of the American Medical Informatics Association* 30(2):308-317.
<https://doi.org/10.1093/jamia/ocac227>
- [30] Eurostat (2026). Individuals: internet activities (isoc_ci_ac_i), indicator L_IUAPR, accessing personal health records online, 2024.
https://ec.europa.eu/eurostat/databrowser/view/isoc_ci_ac_i/default/table (retrieved 2 October 2026).
- [31] World Bank (2026). World Development Indicators: GDP per capita, PPP (current international \$), NY.GDP.PCAP.PP.CD.
<https://data.worldbank.org/indicator/NY.GDP.PCAP.PP.CD> (retrieved 2 October 2026).
- [32] Shrout P E, Fleiss J L (1979). Intraclass correlations: uses in assessing rater reliability. *Psychological Bulletin* 86(2):420-428. <https://doi.org/10.1037/0033-2909.86.2.420>
- [33] Denmark (2009). *Lov nr. 473 af 12. juni 2009 om Grønlands Selvstyre* (Act on Greenland Self-Government), sections 1 and 23 and annex, list I(b). Lovtidende A. <https://www.retsinformation.dk/eli/lt/2009/473> (opened 2 October 2026).
- [34] Greenland (2001). *Landstingsforordning nr. 6 af 31. maj 2001 om patienters retsstilling* (ordinance on patients' legal position).
<https://nalunaarutit.gl/groenlandsk-lovgivning/2001/ltf-06-2001> (opened 2 October 2026).
- [35] Greenland (2024). *Inatsisartutlov nr. 50 af 25. november 2024 om patienters klage- og erstatningsadgang inden for sundhedsvæsenet* (act on patient complaints and compensation in the health service).
https://nalunaarutit.gl/Groenlandsk-lovgivning/2024/Inatsisartutlov-nr-50-af-25_11_2024 (opened 2 October 2026).
- [36] Kosovo (2013). *Law No. 04/L-125 on Health*. Official Gazette No. 13, 7 May 2013; amended by Law No. 08/L-043 (published 11 May 2022) and Law No. 08/L-176 (published 6 July 2023). <https://gzk.rks-gov.net/ActDetail.aspx?ActID=8666> (opened 2 October 2026).
- [37] European Commission, DG SANTE (2026). *Electronic cross-border health services*. Page last modified 2 March 2026.
https://health.ec.europa.eu/ehealth-digital-health-and-care/digital-health-and-care/electronic-cross-border-health-services_en (opened 2 October 2026).

- [38] Commission Implementing Regulation (EU) 2026/2083 of 18 September 2026 on MyHealth@EU. OJ L, 2026/2083, 21 September 2026. CELEX 32026R2083; applies from 26 March 2027 (Article 19); recital 2 describes the earlier exchange as voluntary. Publications Office record and English text: <http://publications.europa.eu/resource/celex/32026R2083> (opened 2 October 2026). EUR-Lex: https://eur-lex.europa.eu/eli/reg_impl/2026/2083/oj/eng
- [39] European Commission, DG SANTE. *MyHealth@EU KPI Monitoring Framework* (eHDSI Monitoring Framework). Transaction data to 30 June 2025; the site states its figures are under revision. <https://experience.arcgis.com/experience/77f459be23e545b48f46a79cfaf19423> (retrieved 2 October 2026).
- [40] eHealth Network (2024). *Guidelines on Patient Summary, Release 3.4*. November 2024. https://health.ec.europa.eu/document/download/e020f311-c35b-45ae-ba3d-03212b57fa65_en?filename=ehn_guidelines_patientsummary_en.pdf (opened 2 October 2026).
- [41] eHealth Network (2024). *Guidelines on ePrescription and eDispensation, Release 3.1*. November 2024. https://health.ec.europa.eu/document/download/b744f30b-a05e-4b9c-9630-ad96ebd0b2f0_en?filename=ehn_guidelines_eprescriptions_en.pdf (opened 2 October 2026).
- [42] Regulation (EU) 2016/679 of the European Parliament and of the Council of 27 April 2016 (General Data Protection Regulation), Articles 15(3) and 20. OJ L 119, 4 May 2016. ELI: <http://data.europa.eu/eli/reg/2016/679/oj>
- [43] United States. *45 CFR 164.524, Access of individuals to protected health information*. Electronic Code of Federal Regulations, version of 1 September 2026. <https://www.ecfr.gov/api/versioner/v1/full/2026-09-01/title-45.xml?part=164§ion=164.524> (opened 2 October 2026).
- [44] Delbanco T, Walker J, Bell S K, Darer J D, Elmore J G, Farag N, et al. (2012). Inviting patients to read their doctors' notes: a quasi-experimental study and a look ahead. *Annals of Internal Medicine* 157(7):461-470. <https://doi.org/10.7326/0003-4819-157-7-201210020-00002>
- [45] Walker J, Leveille S, Bell S, Chimowitz H, Dong Z, Elmore J G, et al. (2019). OpenNotes after 7 years: patient experiences with ongoing access to their clinicians' outpatient visit notes. *Journal of Medical Internet Research* 21(5):e13876. <https://doi.org/10.2196/13876>
- [46] Blease C, McMillan B, Salmi L, Davidge G, Delbanco T (2022). Adapting to transparent medical records: international experience with "open notes". *BMJ* 379:e069861. <https://doi.org/10.1136/bmj-2021-069861>
- [47] Neves A L, Freise L, Laranjo L, Carter A W, Darzi A, Mayer E (2020). Impact of providing patients access to electronic health records on quality and safety of care: a systematic review and meta-analysis. *BMJ Quality and Safety* 29(12):1019-1032. <https://doi.org/10.1136/bmjqs-2019-010581>
- [48] Ammenwerth E, Neyer S, Hörbst A, Mueller G, Siebert U, Schnell-Inderst P (2021). Adult patient access to electronic health records. *Cochrane Database of Systematic Reviews* 2021(2):CD012707. <https://doi.org/10.1002/14651858.CD012707.pub2>
- [49] Mold F, de Lusignan S, Sheikh A, Majeed A, Wyatt J C, Quinn T, et al. (2015). Patients' online access to their electronic health records and linked online services: a systematic review in primary care. *British Journal of General Practice* 65(632):e141-e151. <https://doi.org/10.3399/bjgp15X683941>
- [50] Antonio M G, Petrovskaya O, Lau F (2020). The state of evidence in patient portals: umbrella review. *Journal of Medical Internet Research* 22(11):e23851. <https://doi.org/10.2196/23851>

- [51] Holt M, MacGibbon J, Smith A K J, Broady T R, Davis M D M, Newman C E (2023). Knowledge of Australia's My Health Record and factors associated with opting out: results from a national survey of the Australian general population. *PLOS Digital Health* 2(3):e0000200. <https://doi.org/10.1371/journal.pdig.0000200>
- [52] Carter P, Laurie G T, Dixon-Woods M (2015). The social licence for research: why care.data ran into trouble. *Journal of Medical Ethics* 41(5):404-409. <https://doi.org/10.1136/medethics-2014-102374>
- [53] Grossman L V, Masterson Creber R M, Benda N C, Wright D, Vawdrey D K, Ancker J S (2019). Interventions to increase patient portal use in vulnerable populations: a systematic review. *Journal of the American Medical Informatics Association* 26(8-9):855-870. <https://doi.org/10.1093/jamia/ocz023>
- [54] Tang P C, Ash J S, Bates D W, Overhage J M, Sands D Z (2006). Personal health records: definitions, benefits, and strategies for overcoming barriers to adoption. *Journal of the American Medical Informatics Association* 13(2):121-126. <https://doi.org/10.1197/jamia.m2025>
- [55] Marelli L, Stevens M, Sharon T, Van Hoyweghen I, Boeckhout M, Colussi I, et al. (2023). The European health data space: too big to succeed? *Health Policy* 135:104861. <https://doi.org/10.1016/j.healthpol.2023.104861>
- [56] Martani A, Skovgaard L, Neubert J J, Richter G, Marelli L (2026). The 2025 European health data space regulation: implementation challenges in harmonising opt-out rules for reusing health data. *Health Policy* 174:105764. <https://doi.org/10.1016/j.healthpol.2026.105764>
- [57] OECD (2015). *Health Data Governance: Privacy, Monitoring and Research*. OECD Health Policy Studies. OECD Publishing, Paris. <https://doi.org/10.1787/9789264244566-en>
- [58] OECD (2022). *Health Data Governance for the Digital Age: Implementing the OECD Recommendation on Health Data Governance*. OECD Publishing, Paris. <https://doi.org/10.1787/68b60796-en>
- [59] Oderkirk J (2021). *Survey results: National health data infrastructure and governance*. OECD Health Working Papers. OECD Publishing, Paris. <https://doi.org/10.1787/55d24b5d-en>
- [60] World Health Organization (2021). *Global strategy on digital health 2020-2025*. ISBN 9789240020924. <https://www.who.int/publications/i/item/9789240020924>
- [61] Burris S, Ashe M, Levin D, Penn M, Larkin M (2016). A transdisciplinary approach to public health law: the emerging practice of legal epidemiology. *Annual Review of Public Health* 37:135-148. <https://doi.org/10.1146/annurev-publhealth-032315-021841>
- [62] Tremper C, Thomas S, Wagenaar A C (2010). Measuring law for evaluation research. *Evaluation Review* 34(3):242-266. <https://doi.org/10.1177/0193841X10370018>
- [63] Prainsack B, El-Sayed S, Forgó N, Szoszkiewicz Ł, Baumer P (2022). Data solidarity: a blueprint for governing health futures. *The Lancet Digital Health* 4(11):e773-e774. [https://doi.org/10.1016/S2589-7500\(22\)00189-3](https://doi.org/10.1016/S2589-7500(22)00189-3)
- [64] Kickbusch I, Piselli D, Agrawal A, Balicer R, Banner O, Adelhardt M, et al. (2021). The Lancet and Financial Times Commission on governing health futures 2030: growing up in a digital world. *The Lancet* 398(10312):1727-1776. [https://doi.org/10.1016/S0140-6736\(21\)01824-9](https://doi.org/10.1016/S0140-6736(21)01824-9)
- [65] Hummel P, Braun M, Tretter M, Dabrock P (2021). Data sovereignty: a review. *Big Data and Society* 8(1). <https://doi.org/10.1177/2053951720982012>

- [66] Carroll S R, Garba I, Figueroa-Rodríguez O L, Holbrook J, Lovett R, Materechera S, et al. (2020). The CARE Principles for Indigenous Data Governance. *Data Science Journal* 19:43. <https://doi.org/10.5334/dsj-2020-043>
- [67] Mandl K D, Kohane I S (2020). Data citizenship under the 21st Century Cures Act. *New England Journal of Medicine* 382(19):1781-1783. <https://doi.org/10.1056/NEJMp1917640>
- [68] Steitz B D, Turer R W, Lin C T, MacDonald S, Salmi L, Wright A, et al. (2023). Perspectives of patients about immediate access to test results through an online patient portal. *JAMA Network Open* 6(3):e233572. <https://doi.org/10.1001/jamanetworkopen.2023.3572>
- [69] Cohen I G, Mello M M (2018). HIPAA and protecting health information in the 21st century. *JAMA* 320(3):231. <https://doi.org/10.1001/jama.2018.5630>
- [70] Huckvale K, Torous J, Larsen M E (2019). Assessment of the data sharing and privacy practices of smartphone apps for depression and smoking cessation. *JAMA Network Open* 2(4):e192542. <https://doi.org/10.1001/jamanetworkopen.2019.2542>
- [71] Price W N, Cohen I G (2019). Privacy in the age of medical big data. *Nature Medicine* 25(1):37-43. <https://doi.org/10.1038/s41591-018-0272-7>
- [72] Grande D, Luna Marti X, Feuerstein-Simon R, Merchant R M, Asch D A, Lewson A, et al. (2020). Health policy and privacy challenges associated with digital technology. *JAMA Network Open* 3(7):e208285. <https://doi.org/10.1001/jamanetworkopen.2020.8285>
- [73] Mostert M, Bredenoord A L, Biesart M C I H, van Delden J J M (2016). Big Data in medical research and EU data protection law: challenges to the consent or anonymise approach. *European Journal of Human Genetics* 24(7):956-960. <https://doi.org/10.1038/ejhg.2015.239>
- [74] Scheibner J, Ienca M, Kechagia S, Troncoso-Pastoriza J R, Raisaro J L, Hubaux J-P, et al. (2020). Data protection and ethics requirements for multisite research with health data: a comparative examination of legislative governance frameworks and the role of data protection technologies. *Journal of Law and the Biosciences* 7(1):lsaa010. <https://doi.org/10.1093/jlb/lsaa010>
- [75] United States. 45 CFR 171.103, *Information blocking*. Electronic Code of Federal Regulations, as reproduced by the Legal Information Institute, Cornell Law School. <https://www.law.cornell.edu/cfr/text/45/171.103> (opened 2 October 2026).
- [76] United States. 42 U.S.C. 300jj-52, *Information blocking*. As reproduced by the Legal Information Institute, Cornell Law School. <https://www.law.cornell.edu/uscode/text/42/300jj-52> (opened 2 October 2026).
- [77] US Department of Health and Human Services (2024). 21st Century Cures Act: Establishment of Disincentives for Health Care Providers That Have Committed Information Blocking. Final rule. *Federal Register* 89 FR 54662, 1 July 2024, effective 31 July 2024. Document 2024-13793. <https://www.federalregister.gov/documents/2024/07/01/2024-13793> (record opened 2 October 2026).
- [78] Federal Trade Commission (2024). FTC finalizes changes to the Health Breach Notification Rule. Press release, 26 April 2024. <https://www.ftc.gov/news-events/news/press-releases/2024/04/ftc-finalizes-changes-health-breach-notification-rule> (opened 2 October 2026).
- [79] Federal Trade Commission (2023). FTC enforcement action to bar GoodRx from sharing consumers' sensitive health info for advertising. Press release, 1 February 2023. <https://www.ftc.gov/news-events/news/press->

[releases/2023/02/ftc-enforcement-action-bar-goodrx-sharing-consumers-sensitive-health-info-advertising](#) (opened 2 October 2026).